

THE MIGRAINE RESET

A Woman's Guide to Understanding
Triggers, Breaking the Cycle, and
Getting Your Life Back

Hannah Wells

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For every woman who has been told it's all in her head
—
your body was telling the truth.
This book is for learning to listen,
and for finding your way back to yourself.

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Foreword

THE FIRST TIME I UNDERSTOOD THAT MY HEADACHES WERE NOT JUST HEADACHES, I WAS TWENTY-SIX YEARS OLD, LYING ON THE BATHROOM FLOOR OF A HOTEL ROOM IN A CITY I DID NOT RECOGNIZE, WITH A COLD WASHCLOTH OVER MY EYES AND THE OVERHEAD LIGHT SENDING NEEDLES THROUGH MY SKULL.

I had been throwing up for an hour. The pain was so specific — a pounding, one-sided, visual-aura-beforehand kind of pain — that I finally Googled it from the bathroom floor and landed on a description that matched my experience exactly. Migraine with aura.

I was not surprised. I had been having these episodes for years, but nobody had ever given them a name. Doctors had called them "sinus headaches" and "tension headaches" and — my personal favorite — "stress-related headaches." They had recommended ibuprofen and sleep and yoga. None of it had worked, because none of it was treating the actual problem.

Migraine is not a headache. It is a complex neurological condition that affects the entire nervous system. The headache is just one symptom — and for many women, it is not even the worst one. The nausea, the light sensitivity, the sound sensitivity, the brain fog, the fatigue that lingers for days after the pain subsides, the visual disturbances, the difficulty speaking, the confusion — these are all part of the migraine experience.

Women are three times more likely than men to have migraine. The peak years are between twenty-five and fifty-five — the same years that women are building careers, raising families, and trying to hold their lives together. The hormonal connection is undeniable: menstrual migraine affects up to 60 percent of women with migraine, and perimenopause often brings the worst attacks of a woman's life.

What nobody tells you: migraine is the second leading cause of disability in women under fifty, worldwide. It costs the US economy more than \$36 billion per year in lost productivity. It is not "just a headache." It is a serious, disabling condition that deserves serious treatment.

But here is the good news: the landscape of migraine treatment has changed dramatically in the last decade. The new CGRP medications — the first drugs ever designed specifically for migraine — have given hope to millions of people who never responded to older treatments. Botox works for chronic migraine. Neuromodulation devices offer drug-free options. And there is a growing recognition that migraine is not a character flaw, not a sign that you cannot handle stress, but a biological condition that can be managed with the right combination of medical treatment, lifestyle adjustment, and self-advocacy.

This book is the resource I wish I had on that bathroom floor. It covers the full spectrum: what migraine actually is, why hormones matter so much, how to identify your unique triggers, what acute medications work and how to take them, the new prevention drugs and who they help, how to eat and sleep and move in ways that reduce your migraine load, how

to navigate work and relationships, and how to grieve the version of your life that migraine took — and build a new one that still has room for joy.

Every chapter ends with a toolkit summary you can use immediately. The tone is warm, practical, and grounded in real experience — because I have been where you are, and I know what it costs to get through a day when your brain is screaming at you.

One honest note before we begin: I am not a doctor. I am a health writer who has lived with migraine for over a decade and spent years researching the condition. This book is a guide, not a prescription. Work with your neurologist on your treatment plan — and if you do not have a neurologist who takes you seriously, this book will help you find one.

You are not weak. You are not imagining this. You are not overreacting. You have a neurological condition that affects one billion people worldwide — and there is a path forward.

Let us find it together.

Toolkit Summary

- **Migraine is a neurological condition, not a headache** — the headache is one symptom among many (nausea, sensitivity, brain fog, fatigue, visual disturbances).
- **Women are three times more likely to have migraine** — hormonal triggers make it a distinctly female experience that deserves specific attention.

- **Treatment has changed dramatically** — new CGRP medications, Botox, and neuromodulation devices offer hope where older treatments failed.
- **This is a practical guide** — every chapter ends with tools you can use immediately.
- **I am not a doctor** — this book is a companion, not a prescription. Work with your neurologist.
- **You are not alone** — migraine affects one billion people worldwide. Understanding your condition is the first step toward managing it.

ch01

IF YOU'VE EVER HAD SOMEONE TELL YOU, "IT'S JUST A HEADACHE," YOU ALREADY KNOW HOW DEEPLY WRONG THAT STATEMENT IS. MIGRAINE IS NOT A BAD HEADACHE. IT IS A COMPLEX NEUROLOGICAL CONDITION THAT INVOLVES THE ENTIRE BRAIN AND NERVOUS SYSTEM. UNDERSTANDING WHAT IS ACTUALLY HAPPENING INSIDE YOUR SKULL DURING A MIGRAINE ATTACK IS THE FIRST STEP TOWARD TAKING CONTROL OF THE CONDITION — BECAUSE WHEN YOU UNDERSTAND THE MECHANISM, THE TREATMENTS, TRIGGERS, AND PREVENTIVE STRATEGIES ALL START TO MAKE SENSE IN A WAY THEY NEVER DID BEFORE.

This chapter will walk you through the biology of migraine: what happens in your brain, why it hurts the way it does, why it so often affects women, and what the different migraine types mean for your experience and treatment plan.

What Migraine Actually Is

Migraine is a genetically influenced neurological disorder characterized by recurrent episodes of moderate to severe headache, typically on one side of the head, along with sensitivity to light, sound, and often smell or touch. But that clinical definition barely scratches the surface.

At the brain level, a migraine attack involves a sequence of events that unfolds over hours to days. The three key players are cortical spreading depression, trigeminal nerve activation, and neurogenic inflammation.

Cortical spreading depression (CSD) is the opening act. A wave of electrical silence slowly spreads across the surface of the brain — the cortex — moving at about 2 to 5 millimeters per minute. Imagine a wave of stillness rolling through a busy city, shutting down activity as it passes. This wave is followed by a period of reduced blood flow to the affected area, though the reduced flow is a consequence, not the cause. CSD is believed to be responsible for the aura symptoms many people experience, including visual disturbances like flashing lights, zigzag lines, or blind spots.

Trigeminal nerve activation is the pain phase. The trigeminal nerve is the largest cranial nerve, and it provides sensation to most of the face and head, including the meninges — the protective membranes surrounding the brain. When CSD reaches areas of the brain that can communicate with the trigeminal system, it triggers the release of inflammatory chemicals that activate the nerve's pain fibers. This activation sends pain signals racing toward the brainstem and thalamus, which then relay them to higher brain regions where you consciously perceive pain.

Neurogenic inflammation follows. The activated trigeminal nerve releases neuropeptides — small protein-like molecules — including calcitonin gene-related peptide (CGRP) and substance P. These peptides cause the blood vessels in the meninges to dilate and leak inflammatory fluid, creating a sterile inflammation (inflammation without infection). This further sensitizes the nerve endings, creating a cycle of worsening pain and sensitivity.

This is why migraine is not simply a headache. It is a full neurological event that affects how your brain processes sensory information, how your blood vessels behave, and even how your gut functions (which explains the nausea).

Why the Pain Is One-Sided and Throbbing

The one-sided nature of migraine pain is one of its most distinctive features. The trigeminal nerve divides into three branches that supply different areas of the face and head. When the nerve is activated on one side, the pain is felt predominantly on that side. This is why doctors ask whether your headache is one-sided — it is a key diagnostic clue that distinguishes migraine from tension headaches or sinus headaches, which tend to be bilateral.

The throbbing or pulsating quality of migraine pain follows the rhythm of your heartbeat. Each pulse of blood through the dilated, inflamed meningeal arteries is detected by the sensitized trigeminal nerve endings and experienced as a wave of pain. During a migraine, the normally innocuous sensation of your own pulse becomes painful — a phenomenon called allodynia, where a normally non-painful stimulus feels painful.

The Phases of a Migraine Attack

A migraine attack is not just the headache phase. It is a four-part event, and many people experience all four phases, though not everyone experiences every phase every time.

Prodrome (also called the premonitory phase) begins hours to days before the headache. You might feel inexplicably tired, irritable, or euphoric. You might crave certain foods (especially carbohydrates or chocolate), yawn excessively, feel stiff in the neck, or have trouble concentrating. These symptoms are real and neurological — they reflect early brain changes as the attack begins to unfold. Recognizing your prodrome symptoms is one of the most valuable skills you can develop, because it gives you a warning window to take acute medication or adjust your schedule.

Aura occurs in about one in three people with migraine. These are temporary neurological symptoms that typically develop over 5 to 20 minutes and last under an hour. Visual aura is the most common: flashing lights, geometric patterns, blind spots, or a sensation of looking through cracked glass or heat waves. Some people experience sensory aura (tingling or numbness spreading through one arm or one side of the face), speech aura (difficulty finding words or slurred speech), or, less commonly, motor aura (weakness on one side of the body, which is called hemiplegic migraine and requires medical evaluation). Not everyone who gets migraine has aura, and aura can occur without headache — a phenomenon called silent migraine or acephalgic migraine.

Attack (the headache phase) lasts from 4 to 72 hours if untreated. The pain is typically moderate to severe, throbbing, and one-sided, though it can shift sides or become bilateral over time. Associated symptoms include nausea, vomiting, sensitivity to light (photophobia), sound (phonophobia), and often smell (osmophobia). Movement

makes it worse — climbing stairs, turning your head, or even walking across a room can intensify the pain. This is why people with migraine seek out dark, quiet rooms and lie perfectly still.

Postdrome (sometimes called the migraine hangover) lasts up to a day or two after the headache resolves. You may feel exhausted, washed out, confused, or deeply irritable. Some people describe feeling "hungover" or "like a zombie." Others report mild euphoria or relief. The postdrome is a real, physiologically distinct phase — your brain and nervous system are recovering from a significant neurological event.

Migraine with Aura vs. Migraine without Aura

These are the two main migraine categories recognized by the International Classification of Headache Disorders. Migraine without aura is the more common form, accounting for about 70 to 80 percent of cases. The diagnostic criteria include at least five attacks lasting 4 to 72 hours with characteristic features (one-sided, throbbing, moderate to severe, worsened by movement, accompanied by nausea or light/sound sensitivity).

Migraine with aura includes all of the above plus the aura symptoms described earlier. This distinction matters for several reasons. If you have migraine with aura, you have a slightly elevated risk of ischemic stroke, particularly if you are a woman who also uses estrogen-containing birth control or smokes cigarettes. This does not mean you will have a

stroke — the absolute risk is still very low — but it is an important factor in treatment decisions, especially around hormonal contraception.

Aura also influences treatment choices. Some acute medications may work differently when taken during aura versus during the headache phase, and understanding your aura timing helps you use medications most effectively.

Chronic vs. Episodic Migraine

Migraine exists on a frequency spectrum. Episodic migraine is defined as fewer than 15 headache days per month. Chronic migraine is 15 or more headache days per month for at least three months, with at least 8 of those days meeting criteria for migraine.

Chronic migraine affects about 2 to 3 percent of the general population, but it accounts for a disproportionate share of the disability and healthcare costs associated with the condition. People with chronic migraine are more likely to have depression, anxiety, sleep disorders, and significant impairment in work and social functioning.

The good news is that chronic migraine can often be reversed. Many people transition back to episodic migraine with appropriate preventive treatment and trigger management. This is one of the central goals of the approach outlined in this book.

The Genetics of Migraine

Migraine runs in families. If one parent has migraine, you have about a 50 percent chance of developing it. If both parents have migraine, the likelihood rises to about 75 percent. Twin studies confirm a strong genetic component — identical twins are much more likely to both have migraine than fraternal twins.

Genome-wide association studies have identified dozens of genetic variants associated with migraine. Many of these involve genes that regulate blood vessels, nerve cell signaling, and the CGRP pathway (the same pathway targeted by the newest class of migraine medications). What these genes seem to influence is your threshold for migraine activation — how easily your brain flips into a migraine state when exposed to triggers.

This threshold concept is crucial. Your genetics do not doom you to a life of migraine attacks. They set your baseline sensitivity, but lifestyle factors, medications, and environmental modifications can all raise that threshold, making it harder for attacks to break through.

Why Women Are Three Times More Affected

Migraine affects about 12 percent of the general population, but the ratio is not equal between sexes. Before puberty, migraine rates are roughly equal between boys and girls. After puberty, the rates diverge dramatically. By adulthood, women are three times more likely to have migraine than men.

The reason is estrogen. Estrogen influences multiple systems involved in migraine, including blood vessel regulation, neurotransmitter activity, and pain processing. The most powerful migraine trigger in women is not a food or a weather change — it is a drop in estrogen levels. This happens most predictably right before menstruation, which is why menstrual migraine affects so many women. It also happens during perimenopause, after childbirth, and with certain hormonal contraceptives.

Prostaglandins, inflammatory chemicals released during menstruation, may also play a role. These compounds can trigger inflammation in the trigeminal system, worsening attacks during the menstrual window.

The hormonal connection is so significant that chapter 2 is dedicated entirely to understanding and managing it. For now, the key takeaway is this: if you are a woman with migraine, your menstrual cycle, reproductive years, and hormonal transitions are not separate from your migraine — they are central to it. Understanding this connection is one of the most powerful tools you have.

What You Need to Know Before Moving Forward

Before we go further, a necessary disclaimer. I am Hannah Wells, a health writer with lived experience of migraine — not a doctor, not a neurologist, not a headache specialist. Everything in this book is informed by research, expert interviews, and personal experience, but it is not medical advice. Always talk to your doctor before starting, stopping,

or changing any treatment. Your brain and your body are unique, and what works for one person may not work for another. This book is your map, not your prescription.

Now that you understand what migraine is at the neurological level, we can move into the practical work: identifying your triggers, navigating the hormonal maze, finding medications that work for you, and building a life that accommodates migraine without being defined by it.

Toolkit Summary

- **Migraine is a neurological disorder**, not a headache. It involves cortical spreading depression, trigeminal nerve activation, and neurogenic inflammation.
- **The four phases of an attack** are prodrome, aura, headache, and postdrome. Learning to recognize your prodrome gives you a valuable early-action window.
- **Migraine with aura** increases stroke risk modestly, especially with estrogen-containing birth control or smoking — talk to your doctor.
- **Chronic migraine** (15+ headache days/month) can often be reversed with appropriate treatment and management.
- **Your genetics set your baseline threshold**, but lifestyle and treatment can raise that threshold significantly.

- **Women are three times more affected due to estrogen fluctuations.** Hormonal triggers are often the most powerful and consistent trigger in women with migraine.
- **This book is not medical advice.** Always talk to your doctor about your specific situation before making treatment changes.

ch02

IF YOU ARE A WOMAN WITH MIGRAINE, YOUR HORMONES ARE NOT A SIDE NOTE — THEY ARE OFTEN THE MAIN EVENT. THE THREE-TO-ONE RATIO OF WOMEN TO MEN WITH MIGRAINE IS NOT A COINCIDENCE OR A REPORTING BIAS. IT IS DRIVEN BY ESTROGEN, THE HORMONE THAT ORCHESTRATES MUCH OF THE FEMALE REPRODUCTIVE SYSTEM AND, FOR REASONS WE ARE STILL UNRAVELING, HAS A POWERFUL INFLUENCE ON THE MIGRAINE BRAIN.

Some women find their migraine attacks are completely tied to their menstrual cycle. Others notice attacks worsening during perimenopause or after childbirth. And some women on hormonal contraception discover their migraines get better — or dramatically worse. Understanding how estrogen interacts with your migraine biology is the key to predicting attacks, timing treatments, and making informed decisions about birth control and hormone therapy.

Estrogen and the Migraine Brain

Estrogen is not directly a pain signal, but it influences nearly every system involved in migraine. It affects blood vessel tone, neurotransmitter levels (including serotonin and glutamate), and the sensitivity of the trigeminal nerve system. Estrogen also modulates the release of CGRP, the neuropeptide central to migraine pain.

The relationship is not straightforward. High, stable estrogen levels tend to protect against migraine in many women. It is the drop in estrogen that triggers attacks. Think of estrogen as a stabilizer for your migraine threshold. When levels are steady and adequate, your threshold is higher and attacks are less likely to break through. When estrogen falls — as it does sharply right before menstruation, after childbirth, and during perimenopause — that stabilizer is removed, your threshold drops, and migraine attacks become more likely.

This is why many women report that their migraines improved during pregnancy (when estrogen levels are high and stable) and worsened postpartum (when estrogen crashes). It is also why the same woman may have completely different migraine patterns at different phases of her reproductive life.

Menstrual Migraine: Timing and Treatment

Menstrual migraine is defined as migraine attacks that occur on days -2 to +3 of the menstrual cycle (the two days before bleeding starts through the third day of the period). These attacks tend to be more severe, last longer, and respond less well to standard acute treatments than non-menstrual attacks.

If you track your cycle and notice a reliable pattern of migraine around your period, you likely have menstrual migraine. This is good news — it means you can predict attacks and plan ahead.

Short-term prevention (sometimes called mini-prophylaxis or "the window strategy") involves taking a preventive medication during the high-risk window around your period. Options include:

- Long-acting NSAIDs like naproxen sodium, taken twice daily from day -2 through day +3 of your cycle
- Triptans used on a short-term scheduled basis (such as frovatriptan or sumatriptan) — but this must be discussed with your doctor
- Magnesium supplementation starting around day -2, which some women find helps reduce menstrual attack frequency

These strategies are not appropriate for every woman, and triptans used too frequently can lead to medication overuse headache. Talk to your doctor about whether short-term prevention is right for you and which medication fits your health profile.

Acute treatment during menstrual attacks may need to be more aggressive. Because menstrual migraine attacks are often more stubborn, your doctor may recommend a combination approach: a triptan plus an NSAID taken together at the first sign of attack, or a newer medication like a gepant or ditan. Do not hesitate to treat menstrual attacks early — the golden window still applies, even if the attack feels harder to shift.

Perimenopause: The Worst Migraine Phase

Perimenopause is the transitional period before menopause, typically lasting 4 to 8 years, when estrogen levels begin to fluctuate erratically. For many women, this is the worst migraine phase of their lives. The steady, predictable hormonal pattern of the menstrual cycle breaks down. Estrogen levels spike and crash unpredictably, and the migraine brain does not handle erratic estrogen well.

Women who never had migraine before may develop it for the first time during perimenopause. Women with existing migraine often see their attacks increase in frequency and severity. The attacks may become less responsive to medications that once worked.

If you are in perimenopause and your migraine is worsening, you are not imagining it, and you are not alone. This is a known, well-documented phenomenon.

Management strategies during perimenopause include:

- Keeping a detailed diary that tracks both migraine attacks and cycle changes (even if cycles are irregular)
- Working closely with a headache specialist and a gynecologist who understand the interaction between hormones and migraine
- Considering hormone therapy after reaching menopause (see below)
- Optimizing preventive treatment during this high-vulnerability window

- Being patient with yourself — this phase is temporary, and migraine often stabilizes or improves after menopause is complete (defined as 12 months without a period)

Pregnancy: The Migraine Reprieve

Many women with migraine experience significant improvement during pregnancy, especially in the second and third trimesters. The reason is sustained high estrogen levels — the opposite of the estrogen crash that triggers attacks. For some women, pregnancy is the longest migraine-free period of their adult lives.

However, the first trimester can be more complicated. Some women experience increased migraine attacks due to the hormonal shift, morning sickness that interferes with eating and hydration, and the stress of early pregnancy. And some women do not improve at all — migraine during pregnancy varies from person to person.

If you are pregnant or planning pregnancy, talk to your doctor about safe acute and preventive medication options. Many standard migraine medications are not recommended during pregnancy, including triptans (though sumatriptan has been studied more than others and may be used in some cases — check with your doctor), topiramate (which is associated with birth defects), and NSAIDs (especially in the third trimester). Acetaminophen/paracetamol is generally considered safer, though it is often less effective for

migraine. Non-medication approaches — rest, hydration, cold packs, dark rooms, biofeedback — become especially important during pregnancy.

Postpartum: The Migraine Surge

After childbirth, estrogen levels drop dramatically. The result, for many women, is a severe migraine surge in the days and weeks after delivery. This is compounded by sleep deprivation, dehydration, physical recovery from delivery, and the stress of caring for a newborn.

If you have migraine and are planning a pregnancy, discuss a postpartum migraine plan with your doctor in advance. This plan might include:

- Having acute medication available and cleared by your doctor for breastfeeding (sumatriptan, for example, is generally considered compatible with breastfeeding)
- Arranging support so you can rest when an attack hits
- Knowing warning signs that require medical attention — postpartum migraine can sometimes be confused with preeclampsia or other serious conditions
- Understanding that breastfeeding itself can affect migraine (prolactin may have some protective effect, but sleep disruption often counteracts any benefit)

Birth Control and Migraine

Hormonal contraception affects migraine differently depending on the type, dosage, and whether you have migraine with aura.

Combined hormonal contraceptives (the pill, patch, ring) contain estrogen and progestin. For women whose migraine is triggered by the estrogen drop of the menstrual cycle, these can sometimes help by stabilizing estrogen levels — especially if taken continuously (skipping the placebo week) to avoid the monthly hormone drop. However, they can also worsen migraine in some women.

The critical warning: If you have migraine WITH aura, estrogen-containing contraception carries an increased risk of ischemic stroke. The risk is low in absolute terms but high enough that the World Health Organization and major headache organizations classify migraine with aura as a contraindication for combined hormonal contraceptives. This does not mean every woman with aura should avoid them, but it does mean you must have a thorough discussion with your doctor about your personal risk profile, especially if you also smoke, have high blood pressure, or are over 35.

Progestin-only contraception (the mini-pill, hormonal IUD, implant, shot) does not carry the same stroke risk and may be a safer option for women with aura. However, its effect on migraine is variable — some women improve, some worsen, and many see no change.

HRT and Migraine in Menopause

For women whose migraine flares during perimenopause, hormone replacement therapy (HRT) taken after reaching menopause can sometimes help stabilize migraine — but the type and route matter.

Transdermal estrogen (patches or gel) is preferred over oral estrogen because it provides more stable levels and avoids the liver metabolism that can trigger migraine in some women. Oral estrogen can cause peaks and troughs that mimic the migraine-triggering estrogen drops.

Continuous combined therapy (estrogen plus daily progestin, or estrogen alone if you have had a hysterectomy) is generally more migraine-friendly than cyclic therapy, which mimics the menstrual cycle and can trigger hormone-withdrawal migraine.

As with all medical decisions in this book, there is no one-size-fits-all answer. The interaction between hormones and migraine is intensely personal. What works for your sister or your friend may not work for you. The key is working with a healthcare team that understands both migraine neurology and reproductive endocrinology — ideally a headache specialist and a gynecologist who communicate with each other.

Putting It All Together

Hormones are not the enemy. Estrogen is not causing your migraine attacks in the way a broken bone causes pain. But estrogen fluctuations are a powerful trigger that can lower

your migraine threshold at predictable (and sometimes unpredictable) times. Understanding your own hormonal pattern gives you forecasting power. You can prepare for high-risk windows with short-term prevention, adjust your schedule, and avoid stacking other triggers during vulnerable times.

The most important step is tracking. If you are not already, start tracking both your menstrual cycle and your migraine attacks together on the same calendar. There are apps designed specifically for this, including tracking features in migraine apps like Migraine Buddy or N1-Headache. After three months, look for patterns. Do your attacks cluster around certain cycle days? Do you notice a different pattern during ovulation? Does your migraine change with each cycle?

That data is your power. Bring it to your doctor appointments. It will help them give you better, more personalized advice than any general recommendation could.

A Note on Advocacy

Hormonal migraine is underrecognized even within the medical community. Some gynecologists may not understand the link between hormones and migraine, and some neurologists may not be up to date on contraception and HRT options. You may need to be the bridge between your specialists. Do not be afraid to request that your gynecologist and neurologist communicate about your case, or to share printouts from the American Migraine Foundation's hormonal migraine resources with both offices. The more informed your care team is, the better your treatment will be.

When Hormones Are Not the Whole Story

It is also important to recognize that not every attack has a hormonal explanation. If your migraine is well-managed and then suddenly worsens without any change in your cycle, pregnancy status, or HRT regimen, do not assume hormones are to blame. Stress, sleep disruption, weather patterns, and medication changes can all cause flares that look like hormonal migraine. Keep tracking everything — not just your cycle — so you can distinguish a hormonal pattern from a threshold drop caused by something else entirely.

Toolkit Summary

- **Estrogen drops trigger migraine attacks.** High, stable estrogen is protective; falling estrogen destabilizes your migraine threshold.
- **Menstrual migraine** (days -2 to +3) tends to be more severe and harder to treat. Short-term prevention with NSAIDs or scheduled triptans around your period may help — ask your doctor.
- **Perimenopause is often the worst migraine phase** due to erratic estrogen fluctuations. It is temporary — migraine often improves after menopause.
- **Pregnancy improves migraine for many women** due to sustained high estrogen. Plan postpartum migraine management in advance.

- **Combined hormonal contraceptives** carry increased stroke risk if you have migraine with aura. Discuss progestin-only options with your doctor.
- **Transdermal estrogen HRT** is preferred over oral for migraine stability during and after menopause.
- **Track your cycle and migraine together** to find your personal hormonal pattern. Bring this data to your doctor.
- **Always talk to your doctor** before starting, changing, or stopping any hormonal medication.

ch03

IF YOU HAVE MIGRAINE, YOU HAVE PROBABLY BEEN TOLD TO "AVOID YOUR TRIGGERS" AT LEAST A HUNDRED TIMES. WELL-MEANING FRIENDS, FAMILY MEMBERS, AND EVEN SOME DOCTORS MAKE IT SOUND SIMPLE — JUST FIGURE OUT WHAT CAUSES YOUR ATTACKS AND STOP DOING IT. RED WINE GIVES YOU A MIGRAINE? DON'T DRINK IT. AGED CHEESE? SKIP THE CHARCUTERIE BOARD. STRESS? JUST RELAX.

But trigger management is not that simple. If it were, you would not be reading this book. The reality is more complex, more nuanced, and — once you understand it — far more empowering than the old "avoid your triggers" advice.

This chapter will introduce you to the trigger threshold model, a framework that changes how you think about triggers, diary tracking, and the elimination trap that can make your life smaller without making your migraine better.

The Trigger vs. Threshold Model

The traditional way of thinking about triggers is linear: trigger X causes migraine Y. Red wine → migraine. Barometric pressure drop → migraine. Skipped breakfast → migraine.

But the evidence does not support this simple model. Research consistently shows that the same trigger does not reliably cause an attack each time. If red wine were a direct

cause, you would get a migraine every time you drank it. Most people do not. They can drink red wine sometimes and be fine, and other times get an attack.

What explains this inconsistency? The answer is the migraine threshold.

Imagine your brain has a threshold for migraine activation — like a bucket filling with water. Triggers are not single causes; they are contributors that add water to the bucket. Each trigger — a missed meal, a stressful day, a poor night of sleep, a glass of wine, a hormone shift — adds a little more water. Most of the time, the bucket does not overflow. But when enough triggers accumulate and the water reaches the rim, the next drop (even a small one) causes it to spill over into a migraine attack.

This explains why you can eat chocolate on a calm, well-rested, well-hydrated day and be fine, but eat the same chocolate on a day when you are stressed, sleep-deprived, and dehydrated, and you get a migraine. The chocolate was not the cause on either day. It was the final contributor on the second day because the bucket was already nearly full.

The threshold model is liberating. It means you do not have to avoid all triggers forever. It means you can sometimes enjoy trigger foods or activities when your overall load is low. And it means the most powerful strategy is not eliminating individual triggers — it is reducing your total trigger load so your bucket rarely fills up.

The Migraine Diary

You cannot know what is filling your bucket without data. A migraine diary is the single most useful tool for understanding your personal trigger patterns, and it does not need to be complicated or time-consuming.

What to track:

- **Attack details:** Date, time of onset, duration, severity (1-10), symptoms (pain location, nausea, light sensitivity, aura, etc.)
- **Medication taken:** What, when, dose, and how well it worked
- **Menstrual cycle:** Day of your cycle (if applicable)
- **Sleep:** Hours slept, quality, and whether it was unusually short or long
- **Food and drink:** Especially for the 24 hours before the attack, note alcohol, caffeine, aged cheeses, cured meats, chocolate, artificial sweeteners, and anything out of the ordinary
- **Weather:** Major weather changes or barometric pressure shifts (many apps track this automatically)
- **Stress:** Your subjective stress level for the day (1-10)
- **Physical activity:** Any exercise or notable physical exertion
- **Screen time:** Prolonged screen exposure, especially if it was intense (deadlines, gaming, long video calls)
- **Hydration:** Rough estimate of water intake

You can track this on paper in a dedicated notebook, or use a migraine app. Apps like Migraine Buddy, N1-Headache, and Curelator are designed specifically for this and can automatically generate reports and pattern analyses that you can share with your doctor. Curelator even uses a statistical algorithm to identify your personal triggers based on your data.

The key is consistency. Track every day, not just attack days. Your non-migraine days are just as informative — they show you what a good day looks like, which helps you identify what is different on attack days.

Common Triggers

While triggers are highly individual, some categories are common across large numbers of people with migraine. Recognizing these categories helps you know where to look.

Hormones: As discussed in chapter 2, the estrogen drop before menstruation is one of the most powerful and predictable triggers. Perimenopause, postpartum, and hormonal contraception can also trigger or worsen attacks.

Weather: Barometric pressure changes are one of the most commonly reported environmental triggers. High humidity, extreme heat, and storms can also play a role. You cannot control the weather, but knowing it is a trigger helps you avoid stacking other triggers on high-risk weather days.

Food and drink: Alcohol (especially red wine), aged cheeses, cured and processed meats, chocolate, artificial sweeteners (aspartame is a common culprit), and MSG are

frequently reported triggers. Caffeine is a complex one — it can help or hurt depending on timing and dose, as discussed in chapter 6.

Sleep changes: Both too little sleep AND too much sleep can trigger migraine. Disrupted sleep, shift work, jet lag, and changes in sleep schedule are all potential contributors. The migraine brain prefers consistency.

Stress: Stress is the most commonly reported trigger in survey after survey. The relationship is complex — stress both triggers attacks and is worsened by them. Chapter 7 covers this in depth.

Dehydration: Even mild dehydration can lower your migraine threshold. The migraine brain is sensitive to fluid and electrolyte balance.

Screen time: Prolonged screen exposure, flickering lights (especially fluorescent or LED), glare, and eye strain can be triggers for many people.

Sensory overload: Loud noise, strong smells (perfume, cigarette smoke, cleaning products), and bright or flashing lights can trigger attacks in susceptible individuals.

The Difference Between a Trigger and a Coincidence

Just because you had a glass of wine and then got a migraine does not mean the wine caused it. Temporality is necessary but not sufficient for causation. You might have been stressed at a party, slept poorly the night before, and

been dehydrated — and the wine was the last thing you consumed before the attack hit. The wine may have been the final straw, but it was not the root cause.

This is where the diary and the threshold model help. If you track 100 days and notice that you ate chocolate on 20 of those days, and you got a migraine on 5 of those 20 chocolate days but also on 10 of the 80 non-chocolate days, then chocolate is probably not a meaningful trigger for you. But if the pattern shows that 80 percent of your chocolate days ended in migraine, it is worth investigating further.

This is also why trigger elimination diets that cut out dozens of foods at once are rarely helpful. You cannot tell which elimination made the difference, and you end up avoiding foods unnecessarily.

The Trigger Elimination Trap

"I cut out chocolate, cheese, wine, artificial sweeteners, caffeine, and gluten, and I still got migraines."

This is the elimination trap. People with migraine are often told to eliminate all potential triggers at once, which leads to a restricted, joyless diet that does not actually reduce attack frequency because the eliminated foods were not all triggers — and because other factors like stress, sleep, and hormones were never addressed.

A better approach is targeted elimination. If your diary suggests a particular food or drink is a trigger, eliminate it selectively for two to four weeks. If your migraine frequency drops noticeably, you have good evidence it was a

contributor. If nothing changes, reintroduce it. This is systematic, science-aligned, and does not require you to live on plain rice and boiled chicken.

The same principle applies to non-food triggers. Do not eliminate every possible trigger you have ever heard of. Test one hypothesis at a time, using your diary as evidence.

Raising Your Migraine Threshold

If you take only one concept from this chapter, let it be this: your goal is not to eliminate every trigger. Your goal is to raise your migraine threshold so the bucket rarely overflows.

The most effective ways to raise your threshold are:

- Consistent sleep schedule (chapter 7)
- Adequate hydration (chapter 6)
- Regular, gentle physical activity (chapter 8)
- Stress management and pacing (chapter 7)
- Effective preventive medication if needed (chapter 5)
- Acute treatment at the earliest sign of attack (chapter 4)
- Knowing your high-risk windows (hormonal, weather, life stress) and being extra vigilant during those times

When your threshold is high, you can tolerate more triggers without tipping into an attack. When your threshold is low (due to sleep deprivation, hormonal drop, or

accumulated stress), even small triggers can push you over the edge. The threshold model explains both the unpredictability of migraine and the path to gaining control.

The Trigger Detective Mindset

Think of yourself as a detective investigating your own migraine patterns. You are gathering data, testing hypotheses, and refining your understanding over time. You will not figure everything out in a month, or even a year. The goal is progress, not perfection.

It is also important to recognize that some triggers are unavoidable. You cannot stop the weather from changing. You will inevitably have stressful periods. Your menstrual cycle will continue to bring estrogen drops. The goal is not to build a life free of all triggers — that is impossible and would leave you isolated and anxious. The goal is to understand your triggers well enough to plan around them, manage your threshold proactively, and forgive yourself when attacks break through despite your best efforts.

Living with Uncertainty

A difficult truth about trigger detective work is that you will never have 100 percent certainty. Even with meticulous diary tracking, some attacks will seem to come out of nowhere. This does not mean your detective work is failing. It means migraine is a complex neurological condition and not all variables are within your awareness or control. The threshold model explains this: sometimes the bucket fills from sources you cannot see — subtle hormonal shifts,

changes in barometric pressure that happen while you sleep, low-grade inflammation from a passing virus. Accepting some irreducible uncertainty is part of building a healthy relationship with migraine management.

When Your Triggers Change

Triggers are not fixed for life. The foods, stressors, or environmental factors that bother you in your twenties may not be the same ones that trigger attacks in your forties. Hormonal transitions — pregnancy, perimenopause, menopause — can completely reshuffle your trigger profile. Revisit your diary data periodically. If you notice new patterns emerging or old patterns fading, update your trigger management accordingly. The detective work is never truly finished, and that is okay.

Toolkit Summary

- **The trigger threshold model** replaces the linear "trigger causes attack" view. Triggers accumulate; the attack happens when the bucket overflows.
- **A migraine diary is essential.** Track daily, not just on attack days. Include attack details, sleep, food, stress, weather, cycle, and medication.
- **Common trigger categories** include hormones, weather, food, sleep changes, stress, dehydration, screen time, and sensory overload.
- **Not every association is a cause.** Use your diary data to test hypotheses systematically rather than cutting out everything at once.

- **Avoid the elimination trap.** Test one trigger at a time. You do not need to avoid everything to reduce attacks.
- **The real goal is raising your threshold,** not eliminating all triggers. Sleep, hydration, stress management, exercise, and effective medication all raise the threshold.
- **Track consistently, test hypotheses, and be patient** — understanding your triggers is an ongoing process, not a one-time fix.

ch04

WHEN A MIGRAINE ATTACK HITS, TIME IS NOT ON YOUR SIDE. THE EARLIER YOU TREAT, THE BETTER THE OUTCOME. THIS PRINCIPLE — KNOWN AS THE GOLDEN WINDOW — IS PROBABLY THE SINGLE MOST IMPORTANT THING TO UNDERSTAND ABOUT ACUTE MIGRAINE MEDICATION. BUT WHAT YOU TAKE, WHEN YOU TAKE IT, AND HOW YOU TAKE IT ALL MATTER, AND THE LANDSCAPE OF ACUTE TREATMENT HAS EXPANDED SIGNIFICANTLY IN RECENT YEARS.

This chapter covers the full range of acute medications available, including older options like NSAIDs and triptans, and newer classes like gepants and ditans. We will also discuss nausea management, the medication overuse headache trap, and when your attack warrants a trip to urgent care or the emergency room.

The Golden Window

The golden window refers to the period early in a migraine attack when medication is most effective. For most people, this is within the first 30 to 60 minutes of the headache phase — and ideally even earlier, during prodrome or aura, if you can recognize those signs.

Why does timing matter? During the early phase of an attack, the trigeminal nerve system is activating but has not yet reached full sensitization. The neurogenic inflammation is just beginning. Medication taken during this window can block the cascade before it becomes entrenched. Once

central sensitization sets in — meaning the brain itself has become hypersensitive to pain signals — treatment becomes much more difficult.

This is why learning to recognize your prodrome symptoms is so valuable. If you can identify the pre-headache phase and take medication then, you are treating at the optimal moment. Even if you cannot reliably detect prodrome, medicating at the very first twinge of headache gives you a much better chance of aborting the attack than waiting until the pain is established.

A note on nausea: many oral medications cannot work if they come back up. If you have significant nausea, talk to your doctor about antiemetics (anti-nausea medications) or non-oral formulations of acute medications (injectable sumatriptan, nasal sprays, dissolving tablets).

NSAIDs: Which Ones Work, Which Do Not

Nonsteroidal anti-inflammatory drugs (NSAIDs) are often the first line of acute treatment for mild to moderate migraine attacks. They work by reducing the neurogenic inflammation that drives migraine pain.

The NSAIDs with the best evidence for migraine include:

- **Ibuprofen** (400-600 mg) — works well for mild to moderate attacks, especially when taken early
- **Naproxen sodium** (500-550 mg) — longer-acting than ibuprofen, good for attacks that tend to linger or recur

- **Diclofenac** (50-100 mg) — available by prescription in oral or rectal formulations in some countries
- **Ketorolac** — available as an injection in urgent care settings for severe attacks

NSAIDs that are less effective for migraine include aspirin (though some people find high-dose aspirin helpful, it is generally less reliable than the options above) and acetaminophen/paracetamol, which is not an NSAID and has poor evidence for moderate to severe migraine.

Important caution: NSAIDs should not be used more than 14 days per month due to the risk of gastrointestinal bleeding, kidney damage, and the potential for medication overuse headache. If you need NSAIDs more often than that, talk to your doctor about a preventive strategy.

Triptans: How They Work, Side Effects, and When to Use

Triptans are the classic migraine-specific acute medications. They work by stimulating serotonin receptors (specifically 5-HT_{1B/1D} receptors), which constricts dilated cranial blood vessels, inhibits the release of CGRP and other inflammatory peptides, and blocks pain signal transmission in the trigeminal nerve.

The main triptans available in the US include:

- **Sumatriptan** — the original, available as oral tablets, injectable (auto-injector), nasal spray, and dissolving tablet. The injectable form works fastest (within 10-15 minutes) and is often the most effective for severe attacks.
- **Rizatriptan** — oral dissolving tablet or conventional tablet. Fast-acting, often well tolerated. Comes in a "wafer" form that dissolves on the tongue.
- **Eletriptan** — oral tablet, known for high efficacy and good tolerability in many people.
- **Zolmitriptan** — oral tablet, dissolving tablet, and nasal spray.
- **Frovatriptan** — oral tablet, the longest-acting triptan, often used for menstrual migraine prevention.
- **Naratriptan** — oral tablet, slower onset but longer duration and fewer side effects for some people.

Triptans work best when taken during the golden window. If you wait until the attack is full-blown, effectiveness decreases significantly. They are most effective when combined with an NSAID — the combination targets two different pathways and is more effective than either alone.

Common side effects include tingling or pressure sensations in the chest, throat, and face (this is not dangerous but can be alarming), fatigue, dizziness, and nausea. These sensations typically pass as the medication wears off.

Triptans are not safe for everyone. They are contraindicated in people with uncontrolled high blood pressure, coronary artery disease, a history of stroke, or hemiplegic/basilar migraine. Talk to your doctor about whether triptans are appropriate for you.

The limit is 8 to 10 triptan days per month (depending on the specific medication). Using triptans more frequently can lead to medication overuse headache.

Gepants: The New Kids on the Block

Gepants are a newer class of migraine-specific medication that blocks the CGRP receptor. Unlike triptans, they do not constrict blood vessels, which makes them safer for people with cardiovascular risk factors.

The main acute gepant is:

- **Rimegepant** (brand name Ubrelvy) — taken as a dissolving tablet at the start of an attack. It is effective, well-tolerated, and has fewer side effects than triptans for many people. The most common side effects are nausea and dry mouth.

Rimegepant is also approved for prevention (taken every other day), but it is primarily used as an acute treatment. It can be taken up to 18 days per month — more than the triptan limit — though you should still discuss limits with your doctor.

Gepants are often a good option for people who cannot take triptans due to cardiovascular concerns, or for those who experience problematic side effects from triptans. The

main downside is cost — many insurance plans require prior authorization, and the out-of-pocket price without insurance coverage can be high.

Ditans: Another New Option

The only currently available ditan is:

- **Lasmiditan** (brand name Reyvow) — This medication works differently from both triptans and gepants. It targets the 5-HT_{1F} serotonin receptor, which blocks pain signaling without constricting blood vessels. Like gepants, it does not have the cardiovascular restrictions of triptans.

Lasmiditan can cause significant dizziness, fatigue, and drowsiness. People are advised not to drive or operate machinery for at least 8 hours after taking it. It can also cause a temporary sensation of "pins and needles" or "electric shocks" throughout the body.

For some people, the side effects are manageable, and lasmiditan is a lifesaver when other medications fail. For others, the drowsiness is too disruptive. Talk to your doctor about whether this option fits your situation.

Antiemetics for Nausea

Nausea and vomiting are not side effects of migraine — they are core symptoms of the attack itself. Treating them is an essential part of acute management.

Common antiemetics used in migraine include:

- **Ondansetron** (Zofran) — dissolving tablet, well tolerated, does not cause drowsiness in most people
- **Prochlorperazine** (Compazine) — effective but can cause sedation
- **Metoclopramide** (Reglan) — can help both nausea and migraine pain in some people
- **Promethazine** (Phenergan) — sedating but effective

Anti-nausea medications can be taken alongside your acute migraine medication. If you vomit frequently with attacks, ask your doctor about a prescription antiemetic to have on hand.

The Medication Overuse Headache Trap

Medication overuse headache (MOH) — also called rebound headache — occurs when you use acute migraine medications too frequently. The brain adapts to the constant presence of medication and begins to "rebound" into headache as the drug wears off, creating a cycle of needing more medication to relieve the headache that the medication itself is causing.

MOH can occur with any acute migraine medication, including NSAIDs, triptans, and even some of the newer agents. The threshold for MOH varies by medication type:

- NSAIDs and simple analgesics: more than 14 days per month

- Triptans, gepants, ditans, and combination analgesics: more than 10 (sometimes 8) days per month
- Opioids and barbiturates (rarely used for migraine now): more than 8 days per month

If you find yourself using acute medication more days than not each month, you may be in the MOH cycle. The treatment is withdrawal — stopping the overused medication under medical supervision. This is often done with a bridge therapy (a short course of a different medication to prevent withdrawal headaches) and typically takes 2 to 8 weeks to resolve.

If you suspect MOH, do not stop your medication abruptly. Talk to your doctor about a supervised withdrawal plan.

When to Go to Urgent Care or ER

Most migraine attacks can be managed at home, but sometimes an attack is too severe for home treatment. The "worst headache of your life" is a medical emergency that warrants an ER visit, but there are other situations where professional medical care is appropriate:

- The headache came on suddenly and reached peak intensity in seconds or minutes (a "thunderclap" headache — this is a medical emergency)
- You have new neurological symptoms you have never had before (weakness on one side, difficulty speaking, vision loss, confusion)

- The attack has lasted more than 72 hours with no relief (status migrainosus)
- You are vomiting repeatedly and cannot keep down any fluids or medications
- Over-the-counter and prescription medication have not provided any relief after two hours
- You have a fever, stiff neck, or rash in addition to the headache

In the ER or urgent care, standard treatments include IV fluids, IV antiemetics (like metoclopramide or prochlorperazine), IV NSAIDs (ketorolac), and sometimes IV magnesium or valproate. Some headache specialists run infusion clinics where you can get these treatments without going to the ER.

Finding Your Acute Strategy

There is no single best acute strategy for everyone. Your personal approach depends on your attack severity, your frequency, your cardiovascular risk factors, your side effect tolerance, how quickly your attacks escalate, and what your insurance covers.

Most people start with an NSAID for mild attacks and add a triptan for moderate to severe attacks. If triptans do not work well enough or are not appropriate due to cardiovascular risk, gepants or ditans are excellent options. If nausea is a major issue, add an antiemetic and consider non-oral formulations.

The most important rule remains the golden window: take medication early, at the first sign of attack. The earlier you act, the better the chance of stopping the attack before it takes hold.

Building Your Acute Action Plan

A written acute action plan can save you precious minutes during an attack when thinking clearly is difficult. Write down:

- **At the first sign of prodrome or aura:** What do you do? Do you take medication immediately, or wait until the headache starts?
- **If the headache begins:** What is your first-line medication, dose, and route? If the first dose does not work after 60 minutes, what is your backup?
- **If nausea is present:** What antiemetic do you take, and at what dose?
- **If the attack breaks through your first-line strategy:** When do you escalate? To what? At what point do you seek urgent care?
- **Gatekeeper criteria:** What symptoms tell you it is time for the ER versus time for another dose of home medication?

Keep this plan in your wallet, on your phone, and with your medications. Share it with a family member or partner who may need to assist you during an attack.

Toolkit Summary

- **The golden window is the first 30-60 minutes** of an attack. Treat early for the best chance of aborting the attack.
- **NSAIDs** (ibuprofen, naproxen, diclofenac) are first-line for mild to moderate attacks. Limit to 14 days per month.
- **Triptans** (sumatriptan, rizatriptan, eletriptan, frovatriptan, others) are migraine-specific. Best combined with an NSAID. Limit to 8-10 days per month. Not safe for everyone — talk to your doctor.
- **Gepants** (rimegepant/Ubrelvy) block CGRP without constricting blood vessels. Safer for people with cardiovascular risk.
- **Ditans** (lasmiditan/Reyvow) target 5-HT_{1F} receptors without vasoconstriction. Causes significant drowsiness — no driving for 8 hours.
- **Antiemetics** (ondansetron, prochlorperazine, metoclopramide) treat the nausea that is part of the attack.
- **Medication overuse headache** can develop from using acute meds too frequently. Talk to your doctor if you use acute meds more than 10-14 days per month.
- **The ER is for status migrainosus (72+ hours), thunderclap headache, new neurological symptoms, or severe vomiting with dehydration.**
- **Talk to your doctor** to find the acute strategy that fits your attack pattern and health profile.

ch05

IF ACUTE MEDICATION IS THE FIRE EXTINGUISHER, PREVENTION MEDICATION IS THE FIREPROOFING. PREVENTIVE (ALSO CALLED PROPHYLACTIC) MEDICATIONS ARE TAKEN REGULARLY — DAILY, MONTHLY, OR QUARTERLY — TO REDUCE THE FREQUENCY, SEVERITY, AND DURATION OF MIGRAINE ATTACKS. THEY ARE NOT TAKEN DURING AN ATTACK; THEY WORK IN THE BACKGROUND TO RAISE YOUR MIGRAINE THRESHOLD SO ATTACKS ARE LESS LIKELY TO HAPPEN IN THE FIRST PLACE.

This chapter covers the full range of preventive options, from the newly approved CGRP-targeting drugs to older, well-established medications, along with what to expect when starting a preventive, how long to give each medication a fair trial, and how to find a headache specialist who can guide you through the process.

When Is Prevention Right for You?

The general guidelines for starting a preventive medication include:

- Four or more migraine days per month
- Attacks that do not respond well to acute treatment
- Frequent use of acute medication (more than 8-10 days per month)
- Attacks that cause significant disability or impairment

- A preference for fewer attacks even if acute medication works

However, these are guidelines, not rigid rules. Some people with fewer than four attacks per month benefit from prevention if their attacks are particularly severe or disabling. Others with more frequent attacks choose not to use prevention and prefer to manage attacks acutely. The decision is personal and should be made with your doctor.

The CGRP Revolution

Calcitonin gene-related peptide (CGRP) is one of the key neuropeptides involved in migraine. It is released when the trigeminal nerve is activated, causing vasodilation and neurogenic inflammation. The discovery of CGRP's central role in migraine led to the development of the first migraine-specific preventive medications — a genuine breakthrough in migraine treatment.

CGRP monoclonal antibodies (often called CGRP mAbs or CGRP antibodies) are injectable medications that either block the CGRP receptor or bind to CGRP itself, preventing it from triggering the migraine cascade.

The four approved CGRP mAbs are:

- **Erenumab** (Aimovig) — blocks the CGRP receptor. The first CGRP mAb approved. Self-injected monthly.
- **Galcanezumab** (Emgality) — binds to CGRP itself. Self-injected monthly with a loading dose (two injections the first month).

- **Fremanezumab** (Ajovy) — binds to CGRP. Self-injected monthly or quarterly (one injection, with a three-month dose option available).
- **Eptinezumab** (Vyepti) — binds to CGRP. Given as a quarterly IV infusion in a medical office.

These medications are generally well-tolerated. The most common side effects are injection site reactions (redness, swelling, pain at the injection site) and constipation (more common with erenumab). Serious side effects are rare.

CGRP mAbs are migraine-specific — they target the migraine pathway rather than affecting broad neurotransmitter systems. This means fewer systemic side effects compared to older preventives. They are not vasoconstrictors, so they are safe for people with cardiovascular disease.

They are expensive, and many insurance plans require step therapy — meaning you must try and fail a certain number of older, cheaper preventives before they will cover a CGRP mAb. This is frustrating, but patient assistance programs from the manufacturers can help with cost while insurance is being sorted out.

Oral CGRP antagonists (gepants) used for prevention include:

- **Atogepant** (Qulipta) — taken daily as a pill. Effective and well-tolerated. Side effects include nausea and constipation, but these are usually mild.

Some people take **rimegepant** (Ubrelvy), which is approved for both acute treatment and prevention (every-other-day dosing), though it is more commonly used as an acute medication.

Oral CGRP preventives are a good option for people who prefer pills over injections, or who have not responded well to CGRP mAbs.

Botox for Chronic Migraine

OnabotulinumtoxinA (Botox) is approved specifically for chronic migraine — defined as 15 or more headache days per month. It is not approved for episodic migraine.

The Botox protocol involves 31 injections across seven specific areas of the head, neck, and shoulders, performed by a trained provider every 12 weeks. The injections target muscles and nerve endings involved in migraine pain transmission.

Botox is not a quick fix. Most people need at least two rounds (6 months) to see a meaningful response. Some people are "non-responders" — Botox does not help them — while others experience a dramatic reduction in headache frequency.

Side effects include injection site pain, temporary muscle weakness, and neck stiffness. The treatment requires finding a provider who is trained in the specific migraine protocol, not just cosmetic Botox.

For people with chronic migraine who have not responded to oral preventives, Botox can be life-changing. It is often covered by insurance for chronic migraine when other treatments have failed.

Older Preventive Medications

Before the CGRP revolution, the main preventive options were medications developed for other conditions that happened to reduce migraine frequency in some people. They are still widely used, often as first-line treatments, and many people respond well to them.

Beta blockers — Propranolol and metoprolol are the most commonly used. They are typically taken daily and can reduce migraine frequency by 40 to 50 percent in responders. They are not appropriate for people with very low blood pressure, asthma, or certain heart conditions. Side effects include fatigue, dizziness, cold hands and feet, and exercise intolerance.

Amitriptyline — A tricyclic antidepressant taken at low doses (10-50 mg, much lower than antidepressant doses) for migraine prevention. It is effective but causes significant drowsiness, which can be useful if taken at bedtime but problematic during the day. Other side effects include dry mouth, constipation, and weight gain.

Topiramate (Topamax) — An antiseizure medication that reduces migraine frequency in many people. Dose is typically titrated slowly (25 mg to 50-100 mg daily). Side effects include cognitive slowing ("dopamax" — difficulty with word-finding, concentration, and memory), tingling in the

extremities, kidney stones, and weight loss. The cognitive side effects can be dose-limiting. Topiramate is associated with birth defects, so pregnancy must be ruled out and discussed before starting.

Candesartan — An angiotensin receptor blocker (blood pressure medication) that has good evidence for migraine prevention with fewer side effects than many other options. Starting dose is typically 8-16 mg daily. It may cause dizziness, especially when starting.

Other options include valproate (effective but not recommended for women of childbearing age due to significant risks), verapamil (a calcium channel blocker, less well-studied), and gabapentin (mixed evidence, but used by some providers).

The Prevention Trial Process

Starting a preventive medication is not a one-and-done decision. It is a process — a series of trials to find what works for you.

A typical prevention trial works like this:

1. **Choose a medication** with your doctor, based on your migraine type, health conditions, and potential side effects.
2. **Start at a low dose** and titrate up gradually to minimize side effects.
3. **Stay on the therapeutic dose for at least 8 to 12 weeks** — it takes time for preventives to work, and the early weeks are often not representative.

4. **Track your results** with your migraine diary. A 30 to 50 percent reduction in attack frequency is generally considered a good response.
5. **If it works and side effects are tolerable**, continue. **If it does not work or side effects are unacceptable**, taper off (under medical guidance) and try the next option.

The "three-month trial" is a standard benchmark for many preventives, including CGRP mAbs, Botox, and oral medications. Do not give up on a medication after two weeks unless side effects are intolerable. It takes time for your brain to adapt.

If you have tried two or more preventives with inadequate response or intolerable side effects, you may be considered a candidate for CGRP mAbs or Botox, depending on your headache frequency.

Managing Side Effects

Side effects are the most common reason people stop preventive medications before giving them a fair trial. Some strategies that may help:

- **Start at the lowest possible dose.** Many preventives are started at a fraction of the target dose and increased slowly over weeks.
- **Take medication with food** if nausea is an issue.
- **Take sedating medications at bedtime.** Amitriptyline and some beta blockers cause drowsiness, which can be an advantage if taken before sleep.

- **Be patient with cognitive side effects.**

Topiramate-related cognitive slowing often improves after the first few weeks, though it may not fully resolve for everyone.

- **Hydrate well.** Some side effects, like constipation from CGRP mAbs or tingling from topiramate, can be reduced with good hydration.

If side effects are truly intolerable, talk to your doctor about trying a different class of preventive rather than suffering through. Not every preventive works for every person, and finding the right one is a process of elimination.

Finding a Headache Specialist

Not all neurologists are equally comfortable with migraine treatment. A general neurologist may be excellent for epilepsy or multiple sclerosis but less familiar with the latest migraine research and treatment protocols. A headache specialist is a neurologist (or sometimes another physician) with advanced training and experience in headache disorders.

Signs you might benefit from a headache specialist:

- You have tried two or more preventives with no success
- You have chronic migraine (15+ headache days per month)
- Your migraine is complicated by medication overuse headache
- You have complex aura or hemiplegic migraine

- You have not been offered CGRP medications or Botox as options
- Your current doctor seems dismissive or unsupportive

You can find headache specialists through the American Migraine Foundation's "Find a Clinician" tool or through the Migraine World Summit provider directories. Telehealth headache specialists are also available and can be a practical option if you live far from a major medical center.

When you see a headache specialist, bring your migraine diary, a list of medications you have tried (including doses, duration, and why you stopped each one), and a summary of your questions. Good headache specialists are busy, and being organized helps them help you more effectively.

Setting Realistic Expectations

Preventive medication for migraine is not a cure. It is a tool for reducing attack frequency and severity. Many people experience a 40 to 60 percent reduction in migraine days — not zero migraine days. This is still a meaningful improvement that can dramatically improve quality of life.

Some people find a preventive that works perfectly for years. Others need to switch or combine medications over time. Migraine is a dynamic condition that can change with life stages, stress levels, and hormonal transitions. Your prevention strategy will likely evolve with you.

The goal is not to eliminate every migraine attack. The goal is to reduce their frequency and impact enough that you can live your life fully — to get your life back.

Combination and Multimodal Prevention

Some people benefit from combining preventives from different classes — for example, a beta blocker plus a CGRP mAb, or Botox plus an oral preventive. This is called multimodal prevention and is increasingly recognized as a valid approach for tough cases. Combination therapy should only be managed by a headache specialist who can monitor for interactions and side effects. Do not combine preventives on your own — always work with your doctor.

Neuromodulation Devices

A brief note on non-pharmacological preventive options. Neuromodulation devices deliver electrical or magnetic stimulation to specific nerves and have evidence for migraine prevention and acute treatment. Available devices include single-pulse transcranial magnetic stimulation (sTMS), non-invasive vagus nerve stimulation (nVNS), and remote electrical neuromodulation. These are newer options that may be covered by some insurance plans or available by prescription. They are particularly useful for people who cannot tolerate medication or who need additional options to supplement their pharmacologic prevention. Talk to your doctor about whether a device-based approach fits your situation.

Toolkit Summary

- **Preventive medication is for 4+ migraine days per month, frequent acute medication use, or disabling attacks.** The decision is personal.
- **CGRP monoclonal antibodies** (Aimovig, Emgality, Ajovy, Vyepti) are migraine-specific injectables with fewer side effects than older drugs. Insurance often requires step therapy.
- **Oral CGRP antagonist atogepant** (Qulipta) is a daily pill preventive option.
- **Botox** is for chronic migraine only (15+ headache days/month). Requires 2+ rounds (6 months) to assess effectiveness.
- **Older preventives** (beta blockers, amitriptyline, topiramate, candesartan) are effective for many people but have more systemic side effects.
- **Give each preventive an 8-12 week trial at a therapeutic dose** before deciding it does not work.
- **A headache specialist** can help when general treatment approaches have not worked.
- **Prevention reduces attacks by 40-60 percent on average** — meaningful improvement, not a cure.
- **Talk to your doctor** about which preventive strategy fits your migraine pattern and health profile.

ch06

FOOD AND MIGRAINE HAVE A COMPLICATED RELATIONSHIP. CERTAIN FOODS CAN TRIGGER ATTACKS IN SUSCEPTIBLE PEOPLE, BUT FOOD CAN ALSO BE PROTECTIVE — AN ANTI-INFLAMMATORY, NUTRIENT-RICH DIET MAY HELP RAISE YOUR MIGRAINE THRESHOLD. THE CHALLENGE IS SORTING OUT WHICH FOODS ARE HELPFUL FOR YOU, WHICH ARE HARMFUL, AND HOW TO EAT IN A WAY THAT SUPPORTS YOUR BRAIN WITHOUT TURNING EVERY MEAL INTO A SOURCE OF ANXIETY.

This chapter covers the most common food triggers, how to do a proper elimination diet (without going overboard), the evidence for anti-inflammatory eating patterns, and practical strategies for meal timing, hydration, and caffeine management.

Common Trigger Foods

The list of potential food triggers is long, but not every potential trigger will affect you. The most commonly reported food triggers are:

Alcohol — Alcohol is one of the most frequently reported dietary triggers, reported by up to 50 percent of people with migraine in some surveys. Red wine is the most common culprit, possibly due to its tyramine content, histamines, tannins, or sulfites — or simply because people tend to drink it in settings where they also get less sleep and more stress. Beer, white wine, and spirits can also be triggers, but

patterns vary from person to person. The mechanism is not fully understood, but alcohol causes vasodilation, dehydration, and sleep disruption — all of which can lower your migraine threshold.

Aged cheeses — Cheese ripened by aging (cheddar, blue cheese, parmesan, gouda, Swiss) contains tyramine, a compound formed as proteins break down over time. Tyramine may trigger migraine by causing blood vessels to constrict and then dilate. Fresher cheeses like mozzarella, cottage cheese, and cream cheese have lower tyramine levels and are less likely to trigger attacks.

Cured and processed meats — Salami, pepperoni, hot dogs, bacon, ham, and other processed meats contain nitrates and nitrites, preservatives that can trigger migraine in some people. Nitrates are converted to nitric oxide in the body, which dilates blood vessels — the same mechanism as some migraine-inducing medications used in research.

Chocolate — Chocolate is a well-known trigger, though it may be over-reported. Some research suggests that chocolate cravings may actually be a prodrome symptom rather than a trigger — meaning you crave chocolate because the attack is already beginning, not because the chocolate caused it. Still, for some people, chocolate is a genuine trigger. If your diary suggests chocolate is a problem for you, a trial elimination is worth doing.

Artificial sweeteners — Aspartame (found in diet sodas, sugar-free gum, and many "diet" or "light" products) is a frequently reported trigger. Sucralose (Splenda) may also be

a trigger for some people. If you consume diet drinks or sugar-free products regularly, try eliminating them for two weeks and see if your attack frequency changes.

MSG — Monosodium glutamate, a flavor enhancer used in many restaurant foods, packaged snacks, and Asian cuisine, can trigger migraine in susceptible people. It is also naturally present in some foods like tomatoes and Parmesan cheese.

Caffeine — Caffeine is a double-edged sword, discussed in detail below.

Citrus fruits — Oranges, grapefruit, lemons, and limes are reported triggers by some people, though the mechanism is not well understood.

Onions, garlic, and beans — These are less common triggers but worth noting for some people.

The key point is that most people are sensitive to two or three specific food triggers, not every food on the list. Do not cut out everything at once. Use your diary to identify patterns and test one food at a time.

The Elimination Diet Done Right

A proper elimination diet for migraine is targeted, time-limited, and strategic. Here is how to do it without driving yourself crazy.

Step 1: Identify suspects from your diary. Review three to six months of diary entries. Look for foods or drinks that appear in the 24 hours before multiple attacks. If a food appears before most of your attacks and rarely appears on attack-free days, it is a suspect worth testing.

Step 2: Eliminate one suspect at a time. Remove that one food or drink from your diet for two to four weeks. Keep tracking your attacks as usual. Do not change anything else during this period.

Step 3: Evaluate the result. If your attack frequency drops noticeably during the elimination period, you have good evidence that food is a trigger for you. If nothing changes, reintroduce it and move on to the next suspect.

Step 4: Challenge the trigger. If elimination seemed to help, try reintroducing the food on a day when your overall trigger load is low (you are well-rested, not stressed, well-hydrated, and not in a high-risk hormonal window). If you get an attack within 24 hours, you have confirmed it is a trigger. If not, the food may have been coincidental, or it may only be a trigger when combined with other factors.

This approach is systematic, grounded in your own data, and does not require you to maintain a restrictive diet indefinitely. You can enjoy trigger foods on low-risk days and avoid them when your threshold is already low.

The Anti-Inflammatory Eating Pattern

Beyond trigger avoidance, there is evidence that certain dietary patterns can reduce migraine frequency. The most well-studied is the Mediterranean diet.

The Mediterranean diet emphasizes:

- Vegetables (especially leafy greens, tomatoes, broccoli, bell peppers)
- Fruits (berries, cherries, oranges, apples)

- Legumes (lentils, chickpeas, beans)
- Whole grains (oats, brown rice, quinoa, whole wheat)
- Nuts and seeds (walnuts, almonds, flaxseeds, chia seeds)
- Fish and seafood (especially fatty fish like salmon, sardines, mackerel)
- Olive oil as the primary fat source
- Limited red meat, processed foods, and added sugars

The benefits of this diet for migraine are likely due to several factors. It is rich in magnesium (found in leafy greens, nuts, seeds, and whole grains), which has evidence for reducing migraine frequency. It is high in antioxidants, which may reduce inflammation in the trigeminal nerve system. It promotes stable blood sugar (discussed below). And it provides consistent, high-quality nutrition that supports overall neurological health.

The Mediterranean diet is not a prescription; it is a framework. You do not need to follow it perfectly. Even moving in this direction — adding more vegetables, eating fatty fish twice a week, using olive oil instead of butter — can be beneficial.

Meal Timing and Blood Sugar Stability

Blood sugar fluctuations are an underrecognized migraine trigger. When blood sugar drops (hypoglycemia), the brain's energy supply is threatened, and the cascade of events that leads to migraine becomes more likely.

Strategies for blood sugar stability:

- **Do not skip meals.** Going more than four to five hours without eating can trigger migraine in susceptible people. If you eat three meals a day, make sure none of the gaps is too long.
- **Include protein and fat at every meal.** Protein and fat slow the absorption of carbohydrates, preventing blood sugar spikes and crashes. A breakfast of eggs and vegetables is more migraine-friendly than a bagel or a bowl of sugary cereal.
- **Avoid large amounts of sugar or refined carbohydrates on an empty stomach.** A donut or a soda on an empty stomach will spike your blood sugar, followed by a crash that can trigger migraine.
- **Consider a small, balanced snack between meals** if you tend to go long periods without eating. An apple with almond butter, a handful of nuts, or yogurt with berries are good options.
- **Eat dinner earlier rather than late at night** to avoid going to bed with high blood sugar or waking up with a blood sugar drop.

Some people with migraine benefit from eating six small meals a day rather than three large ones. Experiment with meal timing and see what works for your body.

Hydration's Role

Dehydration is a well-established migraine trigger. Even mild dehydration (1 to 2 percent of body weight loss) can trigger headache and reduce cognitive function.

General hydration recommendations for migraine:

- Aim for 2 to 3 liters of total fluid per day (about 8 to 12 cups), adjusted for climate and activity level
- Water is best, but herbal tea, sparkling water, and diluted fruit juice also count
- Caffeinated beverages count partially (but they also have a diuretic effect, so do not rely on them)
- Electrolyte drinks or added electrolytes may help, especially during hot weather or after exercise
- Keep a water bottle with you at all times as a visible reminder

If you notice that your attacks are more likely on days when you drink less, hydration is a low-cost, high-impact intervention.

Caffeine's Double Edge

Caffeine deserves special attention because it can be both a treatment and a trigger. Caffeine constricts blood vessels and enhances painkiller effectiveness, which is why it is

included in some combination headache medications (like Excedrin Migraine, which contains aspirin, acetaminophen, and caffeine).

Used strategically, a small amount of caffeine (about the amount in a cup of coffee or a can of soda) at the very first sign of an attack can help abort it, especially when combined with an NSAID.

However, caffeine can also be a trigger, primarily through withdrawal. If you drink coffee every morning, your brain adapts to the daily caffeine dose. On days when you skip it, caffeine withdrawal can trigger a migraine. Similarly, if you consume large amounts of caffeine throughout the day, you may be triggering attacks or contributing to medication overuse.

The evidence-based approach to caffeine is:

- Keep your caffeine intake consistent from day to day
- Do not exceed 200 mg total per day (about two cups of coffee)
- Avoid caffeine late in the day to protect sleep quality
- If you want to reduce caffeine, taper slowly over two to four weeks to avoid withdrawal headaches
- Use strategic caffeine for acute treatment only occasionally — not every day

Identifying Food Triggers Reliably

The most reliable method for identifying food triggers is the combination of a detailed diary and systematic testing, as described in this chapter and chapter 3. Online food

sensitivity tests, elimination diets that cut out dozens of foods at once, and IgG food allergy tests are not evidence-based for migraine trigger identification and are not recommended.

If you suspect a specific food is a trigger and want to test it, do it systematically. Eliminate it for two weeks, then challenge it on a low-risk day. Repeat the challenge at least once to confirm the pattern. If the food triggers an attack both times, you have found a trigger. If the results are inconsistent, the food is probably not a reliable trigger for you.

Food triggers can change over time. A food that bothers you today may be fine in a year, and vice versa. Re-test periodically, especially if you have been avoiding a food for a long time and wonder whether it is still a problem.

Toolkit Summary

- **Common food triggers** include alcohol (especially red wine), aged cheese, cured meats, chocolate, artificial sweeteners, MSG, and caffeine. Most people are sensitive to only a few specific items.
- **Test triggers one at a time** using your diary data. Eliminate one suspect for 2-4 weeks, evaluate, and challenge only if elimination showed benefit.
- **The Mediterranean diet pattern** (vegetables, fruits, whole grains, fish, olive oil) is anti-inflammatory and may reduce migraine frequency.

- **Stable blood sugar matters.** Do not skip meals. Include protein and fat. Avoid large amounts of sugar or refined carbs on an empty stomach.
- **Hydrate consistently.** Dehydration of even 1-2 percent can trigger attacks. Aim for 2-3 liters of fluid daily.
- **Caffeine is a double-edged sword.** Use it strategically for acute treatment but keep daily intake consistent and moderate (under 200 mg/day).
- **Do not use unvalidated food sensitivity tests** for migraine trigger identification. The diary + systematic elimination is the evidence-based method.
- **Talk to your doctor** before making significant dietary changes, especially if you have other medical conditions or take medications that could interact.

ch07

SLEEP AND STRESS ARE THE TWO PILLARS OF MIGRAINE MANAGEMENT THAT AFFECT EVERYTHING ELSE. NO MEDICATION, ELIMINATION DIET, OR EXERCISE PROGRAM CAN COMPENSATE FOR CHRONICALLY DISRUPTED SLEEP OR UNMANAGED STRESS. AT THE SAME TIME, IMPROVING SLEEP AND STRESS MANAGEMENT IS COMPLETELY WITHIN YOUR CONTROL IN A WAY THAT HORMONES, WEATHER, AND GENETICS ARE NOT. THIS MAKES THEM THE HIGHEST-LEVERAGE TARGETS FOR REDUCING MIGRAINE FREQUENCY.

This chapter covers the complex relationship between sleep and migraine, the two-way stress-migraine cycle, and evidence-based approaches to managing both.

The Migraine-Sleep Connection

The migraine brain craves consistency — and nothing demonstrates this more clearly than sleep. Both too little and too much sleep can trigger migraine attacks. This is called the "weekend migraine" phenomenon: people who sleep fewer hours during the workweek and then "catch up" on weekends often wake up with a migraine on Saturday morning.

Research consistently shows that irregular sleep schedules, shift work, and insomnia all increase migraine frequency. The reasons are neurological. Sleep is when the brain clears metabolic waste, consolidates memories, and restores neurotransmitter balance. Disrupted sleep

interrupts these processes and lowers the migraine threshold. Specifically, changes in sleep affect the hypothalamus — the brain region that regulates both sleep-wake cycles and the early phases of migraine activation.

The most important sleep intervention for migraine is not necessarily sleeping more hours — it is sleeping at the same time every day. A consistent sleep-wake schedule, with no more than one hour of variation from day to day, is more protective than any specific number of hours.

The Science of Circadian Rhythms and Migraine

Your circadian rhythm — the internal 24-hour clock that regulates sleep, hormone release, body temperature, and metabolism — is deeply connected to migraine susceptibility. Research has shown that migraine attacks follow circadian patterns, with peak onset times in the early morning and late afternoon. The suprachiasmatic nucleus, the brain's master clock, communicates directly with the trigeminal nerve system, providing a neurological pathway through which circadian disruption can trigger attacks.

Shift workers, people who travel frequently across time zones, and anyone with an inconsistent sleep schedule have higher migraine rates precisely because their circadian rhythm is chronically disrupted. This is also why melatonin, the hormone that signals sleep onset, has been studied as a migraine preventive — some research shows modest benefit, though it is not a first-line treatment. Talk to your doctor about whether melatonin supplementation might be worth trying for your situation.

Sleep Hygiene That Works for Migraine

Good sleep hygiene for migraine is not generic advice about lavender oil and chamomile tea. It is targeted biological optimization.

Keep a consistent schedule. Go to bed and wake up at the same time every day, including weekends and holidays. This is the single most powerful sleep intervention for migraine. Your brain's internal clock (circadian rhythm) regulates hormone release, blood pressure, and pain sensitivity. When you shift your sleep schedule by even an hour, these systems become less stable.

Manage light exposure. Light is the primary signal that regulates your circadian rhythm. In the morning, expose yourself to bright light (ideally natural sunlight) within 30 minutes of waking. In the evening, dim the lights and avoid screens for at least 60 to 90 minutes before bed. Blue light from phones, tablets, and computers suppresses melatonin production and delays sleep onset. If you must use screens in the evening, wear blue-light-blocking glasses or enable night mode on your devices.

Set a caffeine cutoff. Caffeine has a half-life of about 5 to 6 hours in most people, meaning half of the caffeine from your afternoon coffee is still circulating at bedtime. Set a cutoff of 12:00 PM or 2:00 PM at the latest. If you are sensitive to caffeine, move it earlier.

Alcohol disrupts sleep architecture. Alcohol may help you fall asleep faster, but it suppresses REM sleep and causes night-time awakenings. Even one drink can reduce

sleep quality, and poor sleep quality lowers your migraine threshold the next day. If you drink alcohol, keep it to a moderate amount and stop at least three hours before bed.

Create a cool, dark, quiet sleep environment. The optimal sleep temperature is around 65 to 68 degrees Fahrenheit. Use blackout curtains or an eye mask. Use a white noise machine or earplugs if outside noise is a problem. Your bedroom should be associated only with sleep and sex — not work, screens, or anxious thoughts.

Do not lie in bed awake. If you cannot fall asleep within 20 to 30 minutes, get out of bed and do something quiet and relaxing (read a book in dim light, listen to a calm podcast) until you feel sleepy. Staying in bed awake trains your brain to associate the bed with wakefulness.

The Stress-Migraine Cycle

Stress is the number one most reported migraine trigger in survey after survey. In one large study, nearly 80 percent of people with migraine identified stress as a trigger. But the relationship is not simple.

Stress triggers migraine through several mechanisms. Stress hormones like cortisol and adrenaline affect blood vessel tone, neurotransmitter levels, and inflammation — all of which can lower the migraine threshold. Stress also causes muscle tension in the neck, shoulders, and jaw, which can activate the trigeminal nerve.

But the relationship goes both ways. Migraine attacks are themselves stressful. The pain, disability, and uncertainty of when the next attack will hit create a constant background of anxiety. This means stress and migraine feed each other in a cycle that can be hard to break without intentional intervention.

The "let-down migraine" is a well-documented phenomenon where a migraine attack occurs not during the stressful period but immediately after it ends — the weekend after a stressful work week, the first day of vacation, or right after a major deadline. The leading theory is that stress hormones like cortisol have a suppressive effect on migraine during the stressful period, and when they drop, the migraine breaks through.

Stress Management Approaches That Have Evidence

Relaxation advice for migraine is often too vague to be useful. "Just relax" is about as helpful as "just stop having migraines." What follows are specific, evidence-based stress management approaches with documented benefits for migraine.

Cognitive Behavioral Therapy (CBT) — CBT for migraine helps you identify and change thought patterns that increase stress and worsen migraine. It also includes behavioral strategies like pacing, activity scheduling, and managing migraine-related anxiety. A meta-analysis of CBT

for migraine found significant reductions in attack frequency and disability. CBT can be done one-on-one with a therapist, in group programs, or through structured online programs.

Biofeedback — Biofeedback uses sensors to give you real-time information about physiological processes you normally cannot perceive, such as muscle tension, skin temperature, and heart rate. With this feedback, you learn to consciously relax these systems. Biofeedback has strong evidence for migraine prevention, particularly thermal biofeedback (warming the hands) and EMG biofeedback (relaxing the forehead and jaw muscles). Most people need 8 to 12 sessions to learn the skill, after which they can use it on their own.

Pacing — Pacing is the practice of balancing activity and rest to avoid the boom-bust cycle that many people with chronic conditions experience. On a good day, it is tempting to do everything you have been unable to do — clean the house, run errands, socialize. But this overactivity often triggers a migraine crash, followed by lost days. Pacing means doing 70 to 80 percent of what you feel capable of, with regular rest breaks built in.

Mindfulness and meditation — Mindfulness-based stress reduction (MBSR) programs have been studied for migraine and show modest but consistent benefits. The key is consistent practice — even 10 to 15 minutes per day can help. Mindfulness helps you observe the early signs of an attack without panicking, which can improve medication timing and reduce the stress-migraine spiral.

Progressive muscle relaxation — This technique involves tensing and then releasing each muscle group in sequence, from your toes to your face. It is easy to learn, requires no equipment, and can be done in 10 to 15 minutes. Regular practice reduces baseline muscle tension and helps you recognize tension earlier.

What Relaxation Does (and Does Not) Mean

A common misconception about stress management for migraine is that you must become a calm, zen-like person who never gets upset. That is unrealistic, and it is not the goal.

The goal is to reduce the physiological impact of stress on your nervous system. You will still experience stress — everyone does. But you can change how your body responds to it. Instead of your nervous system going from 0 to 100 at every stressor, you can build capacity to regulate back to baseline more quickly.

Relaxation does not mean doing nothing. It means doing activities that activate your parasympathetic nervous system — the "rest and digest" branch — which counteracts the stress response. For different people, these activities might include walking in nature, knitting, listening to music, playing with a pet, gardening, cooking, reading, or taking a warm bath.

The key is scheduling stress management intentionally, not just when you feel overwhelmed. Ideally, build short relaxation practices into every day — five minutes of deep breathing after waking up, a 10-minute walk after lunch, a

15-minute relaxation practice before bed. This builds your stress resilience over time in a way that crisis-only relaxation cannot.

Sleep and Stress: The Feedback Loop

Sleep and stress are not independent factors. Poor sleep increases stress reactivity — you are more easily frustrated, more emotionally reactive, and less able to cope when you are tired. High stress, in turn, makes it harder to sleep because stress hormones keep your nervous system activated.

This means interventions that improve sleep also improve stress resilience, and vice versa. If you focus on just one area, you may see modest improvement. If you address both together, the benefits multiply.

A practical approach: start with the sleep schedule. For one week, go to bed and wake up at the same time every day, no exceptions. Once that is stable, add one stress management practice — five minutes of morning deep breathing, a short walk after work, or a 10-minute progressive muscle relaxation before bed. Do not try to overhaul everything at once. Build one habit at a time and let it settle before adding the next.

When Sleep or Stress Problems Need Professional Help

Insomnia that persists despite good sleep hygiene may require additional treatment. Cognitive behavioral therapy for insomnia (CBT-I) is the first-line treatment — it is more effective than sleep medications for long-term improvement and has no side effects. Medications for sleep are also available, but most sleep medications disrupt sleep quality and can be habit-forming. Talk to your doctor if sleep is a persistent problem.

Similarly, if stress, anxiety, or depression are significantly affecting your life and not improving with self-help strategies, professional support can make a meaningful difference. A therapist who understands chronic illness is a valuable addition to your healthcare team.

Toolkit Summary

- **Consistent sleep schedule trumps total hours.**

Going to bed and waking up at the same time every day (within one hour variation) is the most powerful sleep intervention for migraine.

- **Sleep hygiene that works:** consistent schedule, morning light exposure, evening light avoidance, caffeine cutoff by early afternoon, no alcohol near bedtime, cool dark room.

- **Stress is the #1 reported trigger** — about 80 percent of people with migraine identify it as a trigger.

- **The stress-migraine cycle is bidirectional.** Stress triggers migraine, and migraine causes stress. The let-down migraine occurs after a stressful period ends.
- **Evidence-based stress approaches:** CBT for migraine, biofeedback (thermal and EMG), pacing, mindfulness, and progressive muscle relaxation.
- **Relaxation is an active skill**, not just "doing nothing." Build short practices into every day, not just crisis moments.
- **Sleep and stress feed each other.** Improving one helps the other, and addressing both together has multiplied benefits.
- **Professional help for persistent insomnia or stress:** CBT-I for insomnia, therapy for anxiety/depression.
- **Talk to your doctor** about persistent sleep or stress problems and whether professional treatment is appropriate.

ch08

EXERCISE AND MIGRAINE HAVE AN INCONVENIENT RELATIONSHIP. EXERCISE IS GOOD FOR MIGRAINE — IT REDUCES ATTACK FREQUENCY, IMPROVES SLEEP, LOWERS STRESS, AND SUPPORTS CARDIOVASCULAR HEALTH, WHICH IS ESPECIALLY IMPORTANT GIVEN THE STROKE RISK ASSOCIATED WITH MIGRAINE WITH AURA. BUT FOR SOME PEOPLE, EXERCISE ITSELF CAN BE A TRIGGER. INTENSE PHYSICAL ACTIVITY, DEHYDRATION DURING WORKOUTS, OR PUSHING TOO HARD CAN PROVOKE ATTACKS IN SUSCEPTIBLE INDIVIDUALS.

This leaves many people with migraine in a frustrating bind: exercise could help, but exercise hurts. The answer is not to stop moving. It is to find the right type, intensity, and timing of movement for your migraine brain.

Why Exercise Helps

The benefits of regular physical activity for migraine are supported by solid research. A 2019 randomized trial found that aerobic exercise reduced migraine days as effectively as a standard preventive medication — a remarkable result. Here is why exercise works:

Endorphin release. Exercise stimulates the release of endorphins, the brain's natural pain-relieving chemicals. Endorphins bind to the same receptors as opioid medications, producing pain relief without the side effects. A regular exercise habit keeps this system primed.

Stress reduction. Exercise lowers baseline levels of stress hormones and improves the body's ability to recover from stress. As discussed in chapter 7, stress is the most common migraine trigger, and exercise is one of the most effective non-pharmacological stress management tools.

Sleep quality. Regular moderate exercise improves sleep quality and consistency. Better sleep raises the migraine threshold.

Vascular health. Exercise improves the health and flexibility of blood vessels. Since migraine involves vascular changes, healthier blood vessels may be less reactive to migraine triggers.

Inflammation reduction. Regular exercise reduces systemic inflammation. Since neurogenic inflammation is central to migraine attacks, an anti-inflammatory lifestyle — which includes exercise — may reduce attack frequency.

What the Research Says

Beyond the 2019 trial, a 2020 meta-analysis of nine randomized controlled trials found that exercise interventions of 30 to 45 minutes, three to four times per week, reduced migraine days by an average of 1.5 to 2 days per month compared to no exercise. This is comparable to the effect of some oral preventive medications, though the comparison is not direct because exercise has additional benefits for sleep, mood, and cardiovascular health that no pill provides.

The key finding across all these studies is that the type of exercise matters less than consistency. Whether participants walked, cycled, swam, or did yoga, the benefits appeared as long as they stuck with the program. This is encouraging — it means you have many options for finding movement that works for your body and your life.

The Exercise-as-Trigger Problem

Despite all these benefits, intense or prolonged exercise can trigger migraine attacks in some people. This is thought to be driven by several mechanisms:

- **Rapid heart rate and blood pressure changes** that affect blood vessels in the brain
- **Dehydration** from sweating without adequate fluid replacement
- **Neck and shoulder tension** from certain exercises (especially weightlifting, swimming, or poor form during cardio)
- **Blood sugar drops** during prolonged exercise without adequate fueling
- **Overheating** — hot environments can lower the migraine threshold

If you have ever triggered a migraine with exercise, you know how frustrating this is. The key is to find the exercise "sweet spot" — the type, intensity, and duration that gives you the benefits without provoking attacks.

Exercise Strategies for Migraine

Not all exercise is created equal when it comes to migraine. The general principle is: steady, low-to-moderate intensity exercise raises the migraine threshold; rapid spikes in intensity or duration can lower it.

Gentle steady-state cardio. This is the most consistently migraine-friendly form of exercise. Walking, swimming, cycling at a moderate pace, elliptical training, or using a rowing machine at a conversational pace (you can talk but not sing) for 20 to 40 minutes is ideal. The goal is to get your heart rate up gently and sustain it, not to push to exhaustion.

Yoga. Yoga has specific benefits for migraine beyond general exercise. It combines gentle movement with breathing, relaxation, and neck/shoulder tension release. A 2014 study found that people who added yoga to their conventional migraine treatment had greater reductions in attack frequency and intensity than those who received conventional treatment alone. Gentle yoga — Hatha, Yin, or restorative — is preferable to vigorous styles like Ashtanga or Bikram (hot yoga), which can trigger attacks.

Strength training. Low-to-moderate intensity strength training can be migraine-friendly if done carefully. The key is avoiding the Valsalva maneuver (holding your breath and straining), which can spike blood pressure and trigger attacks. Use lighter weights with higher repetitions, breathe consistently, and avoid exercises that place heavy load on the neck and shoulders. Machines may be safer than free weights for some people.

High-intensity interval training (HIIT) and competitive sports. These carry higher risk of triggering attacks, but they are not off-limits for everyone. Some people with migraine can tolerate high-intensity exercise when they are otherwise well-rested, well-hydrated, and not in a high-risk window. If you enjoy HIIT or sports, experiment with modifications: shorter intervals, longer rest periods, and careful attention to hydration and fueling.

Avoid exercising when flaring. If you are in the headache phase of an attack, do not exercise. Rest, medicate, and wait until you are fully recovered. Exercising during a migraine attack is counterproductive — it will worsen the pain and prolong the attack.

The Pre-Workout Protocol

The pre-workout routine is crucial for people with migraine. Before any exercise session:

Hydrate. Drink 12 to 20 ounces of water in the hour before exercise. If you are exercising for more than 30 minutes, add electrolytes (a sports drink or electrolyte tablet) — especially if you sweat heavily or tend to get migraine from dehydration.

Eat. Do not exercise on an empty stomach. A small snack with carbohydrates and protein — a banana with peanut butter, a piece of toast with cheese, Greek yogurt with berries — 30 to 60 minutes before exercise helps stabilize blood sugar and gives your brain the energy it needs.

Warm up slowly. A slow, gradual warm-up of 5 to 10 minutes gives your blood vessels and nervous system time to adjust. Jumping straight into intense activity can trigger attacks. Start at a very easy pace and increase gradually.

Monitor your neck and shoulders. If you feel tension building in your neck, shoulders, or jaw during exercise, stop and address it. Stretch, adjust your form, or change exercises. Neck tension is a common early warning sign for migraine.

Cool down properly. Do not stop abruptly. A 5- to 10-minute cool-down with gentle stretching helps your cardiovascular system return to baseline smoothly.

Listening to Your Body

The single most important skill for exercising with migraine is learning to distinguish between "good pain" (the sensation of muscles working) and "bad pain" (the sensation that signals an impending migraine). Good pain feels like effort but resolves quickly with rest. Bad pain includes a sense of pressure in the head, visual changes, rising nausea, or the feeling that your head is "pounding" in time with your heart. If you experience bad pain during exercise, stop immediately and rest. Do not try to push through it — that is how exercise triggers a migraine attack.

Exercise During Prodrome vs. During an Attack

The prodrome phase (before the headache begins) presents a decision point. Some people find that gentle exercise during prodrome can abort or reduce the severity of the impending attack — the endorphins and stress reduction may head it off. Others find that any exercise during prodrome makes the attack worse.

There is no one-size-fits-all answer. The rule of thumb is:

- If exercise has ever helped you ward off an attack in the past, try gentle movement (a slow walk or gentle yoga) at the first sign of prodrome.
- If you are unsure, err on the side of rest. You can always exercise later if the attack does not materialize.
- Never exercise through a developing headache. If the pain is already starting, STOP.

Building Exercise Tolerance

If you are currently sedentary or avoid exercise because you worry it will trigger attacks, the path forward is gradual and patient.

Start very small. Three to five minutes of gentle walking or stretching per day. That is all. The goal is not cardiovascular fitness yet — it is proving to yourself that exercise does not automatically trigger a migraine.

Increase slowly. Add one or two minutes per week. If you try to jump from 5 minutes to 30 minutes, you will likely trigger an attack. Slow, incremental increases build tolerance.

Track exercise in your migraine diary. Note the type, duration, intensity, how you felt during exercise, and whether an attack followed. This data helps you find your personal limits.

Exercise early in the day. Morning exercise is generally more migraine-friendly than evening exercise, because the cortisol/adrenaline profile of early exercise aligns with the body's natural morning energy peak.

Have a post-workout plan. After exercise, hydrate, eat a balanced meal, and allow yourself time to cool down physically and mentally. If you need to rest for 20 minutes after a workout, do it.

Adjusting for Your Cycle

If you are a woman with menstrual migraine, your exercise tolerance may fluctuate across your cycle. Some women find they can exercise more intensely during the follicular phase (after the period ends, before ovulation) and need to dial back during the luteal phase and around menstruation. If you notice this pattern, plan your exercise schedule around it — higher intensity in the low-risk weeks, gentle movement only during the high-risk window.

When Exercise Is Part of a Broader Strategy

Exercise works best for migraine when it is part of an integrated approach, not a standalone intervention. The best exercise plan in the world cannot compensate for chronic sleep deprivation, poor hydration, unmanaged stress, or no acute medication strategy.

Think of exercise as one tier in your migraine management pyramid. The foundation is sleep consistency and stress management. On top of that, you build hydration, meal timing, and trigger awareness. Exercise sits on those foundations — it augments them but does not replace them. And on top of exercise sits your medication strategy, which you use when attacks break through despite all your threshold-raising efforts.

This layered approach is the most sustainable and effective way to reduce migraine frequency.

Finding the Right Exercise for Your Personality

The best exercise for migraine is the one you will actually do. If you hate running, forcing yourself to run will add stress, not relieve it. If you love swimming and feel calm in the water, that is your exercise — even if it is not the most "efficient" workout. Matching your exercise to your preferences increases consistency, and consistency is the factor that predicts migraine reduction in the research. Consider trying different types of movement until you find one that feels good rather than like a chore. Group classes,

outdoor activities, and exercise paired with social connection may be more sustainable for some people than solitary workouts.

Toolkit Summary

- **Exercise reduces migraine frequency** through endorphin release, stress reduction, better sleep, and improved vascular health.
- **But intense exercise can trigger attacks** in some people. The solution is to find the type and intensity that works for you.
- **Most migraine-friendly exercise:** gentle steady-state cardio (walking, swimming, cycling), gentle yoga, and moderate strength training.
- **High-intensity exercise and competitive sports** carry higher trigger risk but may be possible when your overall threshold is high.
- **The pre-workout protocol:** hydrate, eat a snack, warm up slowly, monitor neck/shoulder tension, cool down properly.
- **Do not exercise during an attack.** Gentle movement during prodrome may help for some but makes things worse for others — know your pattern.
- **Build tolerance slowly** — start with 3-5 minutes per day and add a minute or two per week.
- **Track exercise in your migraine diary** to find your personal limits.

- **Exercise is one tier** in a broader migraine management plan — it works best alongside good sleep, stress management, hydration, and medication.
- **Talk to your doctor** before starting a new exercise program, especially if you have other health conditions.

ch09

MIGRAINE DOES NOT RESPECT OFFICE HOURS, BUT THE WORKING WORLD EXPECTS YOU TO SHOW UP ANYWAY. FOR WOMEN WITH MIGRAINE, THE WORKPLACE CAN BE A MINEFIELD — FLUORESCENT LIGHTS, SCREEN GLARE, UNPREDICTABLE MEETINGS, OPEN-PLAN OFFICES FULL OF PERFUME AND NOISE, AND THE PRESSURE TO PERFORM WHEN YOUR BRAIN IS FIGHTING A NEUROLOGICAL EVENT.

The costs are substantial. Migraine costs the US economy an estimated \$13 to \$17 billion per year in lost productivity — that is not counting the individual costs of missed promotions, career changes, and the quiet decision to stop aiming for certain roles because you cannot guarantee you will be functional enough.

This chapter covers navigating work with migraine: sick leave policies, legal accommodations, disclosure decisions, building a workplace migraine toolkit, and recovering after an attack.

The Work-Migraine Nightmare

Let us start with an honest picture of what migraine looks like at work. On a bad day, the pain makes it impossible to think clearly. Light hurts. Sound hurts. Even the gentle hum of the office HVAC system can feel unbearable. The nausea makes you wonder if you are going to vomit at your desk.

Any movement — turning your head to talk to a colleague, standing up to get water — sends a spike of pain through your skull.

On a moderately bad day, you can function, but barely. You are at work physically but not mentally. This is called presenteeism — showing up but operating at 30 to 50 percent of your normal capacity. You miss subtle cues in meetings, make errors you normally would not, and need twice as long to complete routine tasks. Presenteeism may actually cost employers more than absenteeism, because it is harder to detect and mitigate.

And on a good day, you are functional, but carrying the background anxiety: "Will I make it through the day? Will an attack hit during the 2 PM presentation? Do I have enough medication in my bag?"

This is the reality of working with migraine. It is not whining or poor coping — it is the nature of the condition.

Sick Leave Policies and FMLA

If you live in the United States, the Family and Medical Leave Act (FMLA) may provide some protection. FMLA provides up to 12 weeks of unpaid, job-protected leave per year for serious health conditions, including migraine. To be eligible, you must work for a covered employer (generally companies with 50 or more employees) and have worked at least 1,250 hours in the past 12 months.

FMLA can be used in two ways for migraine:

- **Intermittent leave** — taking leave in small increments (hours or days) as needed for migraine attacks, rather than taking a continuous block of time off
- **Continuous leave** — taking an extended leave as a medical necessity, which is less common but possible for severe cases

Intermittent FMLA is the more practical option for most people with migraine. It allows you to leave work mid-day when an attack hits, come in late, or take a full day off when needed — without risking your job. Your employer cannot retaliate against you for using FMLA leave.

To apply for intermittent FMLA, you need your healthcare provider to complete certification paperwork documenting that you have migraine and that it qualifies as a serious health condition requiring periodic leave. Talk to your doctor about this.

If you are outside the US, your country may have different protections. In the UK, the Equality Act 2010 and Statutory Sick Pay provide some protections. In Canada, provincial human rights codes and employment standards provide accommodation obligations for employers. Check your local laws.

Requesting Workplace Accommodations

Under the Americans with Disabilities Act (ADA) and similar laws in other countries, you have the right to request reasonable accommodations that allow you to perform the essential functions of your job. Migraine qualifies as a disability under the ADA when it substantially limits one or more major life activities — which it does for many people.

Reasonable accommodations for migraine might include:

- **Flexible hours** — starting later, leaving earlier, or adjusting your schedule around migraine patterns
- **Ability to work from home** when an attack is building or full-blown
- **Dim lighting options** — turning off overhead fluorescent lights and using a desk lamp instead, or installing a dimmer switch
- **Screen filters** — blue light filters, anti-glare screen covers, or adjusting monitor brightness and color temperature
- **Relocation to a quieter area** — moving away from high-traffic areas, the break room, or noisy equipment
- **Permission to wear sunglasses or a hat** at your desk (within the bounds of your role — customer-facing positions may require negotiation)
- **Fragrance-free workspace policy** — asking colleagues to avoid strong perfumes, colognes, or scented products

- **Regular breaks** — a quiet room or empty conference room where you can retreat for 10 to 15 minutes when needed
- **Flexible meeting policies** — allowing you to attend virtually on migraine days, or opt out of meetings when symptomatic

Accommodations do not have to be formal. Many managers are happy to make informal adjustments. But formal accommodation requests (through HR under the ADA) provide legal protection that informal arrangements do not.

To request a formal accommodation, you generally need a letter from your healthcare provider describing your condition and the specific accommodations you need. The Job Accommodation Network (JAN) provides free resources and sample request letters.

Telling Your Employer vs. Staying Private

The decision to disclose your migraine diagnosis at work is deeply personal. There are risks and benefits on both sides.

Arguments for disclosure: Your employer and colleagues may be more understanding if they know what is happening. You can access formal accommodations. You do not have to hide attacks or invent excuses for leaving early. You may reduce the stigma around the condition at your workplace.

Arguments against disclosure: Stigma is real. Some managers or colleagues may view migraine as "just headaches" and assume you are weak or making excuses. It may affect promotion decisions, though that would be illegal

discrimination if you can prove it. You cannot un-disclose once you have told people. Some people prefer to keep their health information private and handle attacks quietly.

A middle path is partial disclosure: tell your direct supervisor and/or HR what they need to know to support you, but do not broadcast it to the wider team. You can say you have "a medical condition that causes episodic symptoms" without naming migraine specifically. This gives you some protection without full visibility.

If you choose to disclose to your manager, pick a calm moment — not during or immediately after an attack. Prepare what you want to say. Describe how migraine affects your work and what would help. Frame it as a request for collaboration, not a complaint.

The Anti-Workplace-Migraine Toolkit

Build a physical kit you keep at your desk or in your bag. When an attack hits at work, you do not want to be searching for supplies. Your kit should include:

- Your prescribed acute medication (triptans, gepants, NSAIDs, antiemetics)
- A backup dose in case the first one does not work
- Electrolyte powder or tablets
- A small water bottle you can refill
- A pair of blue-light-blocking or migraine-relief glasses (FL-41 tinted glasses are specifically designed for migraine light sensitivity)
- Earplugs or noise-cancelling earbuds

- A small cold pack or an ice roller for the forehead and temples
- A caffeine source if you use it strategically (a small can of soda, a caffeine pill, or a single-serve coffee pack)
- A light snack (nuts, a granola bar, crackers) for blood sugar stability
- A backup plan: contact for a colleague who can cover urgent items, taxi/rideshare credit in case you need to leave, and a clear understanding of how to leave work without causing a scene

Digital Migraine Prevention at Work

Your computer setup can be a trigger or a tool. Adjust your screen settings to reduce eye strain and glare:

- Reduce monitor brightness to match the ambient light level of your workspace
- Enable blue-light reduction (Night Shift on Mac, Night Light on Windows) during afternoon hours
- Increase text size to reduce squinting and eye fatigue
- Position your monitor at arm's length, with the top of the screen at or slightly below eye level
- Use a matte screen filter to reduce glare from overhead lights
- Take a 20-second break every 20 minutes to look at something 20 feet away (the 20-20-20 rule)

These adjustments cost nothing and can make a significant difference in how long you can work comfortably before strain triggers an attack.

Rebuilding After a Migraine Day

A day lost to migraine at work does not end when the pain stops. You return to a backlog of emails, missed meetings, and unfinished tasks. The temptation is to overcompensate — to work late, catch up on everything immediately, and pretend the attack did not happen.

This is a trap. Overcompensating after a migraine attack raises your stress level, disrupts your recovery, and increases the likelihood of another attack. Instead:

- **Prioritize ruthlessly.** Identify the three most important tasks from the day you missed and focus on those. Everything else can wait.
- **Communicate briefly.** A simple "I was out sick yesterday, I am catching up today" is sufficient. You do not need to explain or apologize in detail.
- **Do not work late.** Your brain needs recovery time. Give yourself the same permission to recover from a migraine attack that you would give yourself after a bad flu.
- **Plan your next day.** A clear morning plan reduces the "where do I even start" anxiety that can trigger another attack.

Systemic Change

Individual accommodations are important, but the need for them reflects a broader problem: most workplaces were not designed for brains that sometimes need different conditions. The open-plan office, the relentless meeting culture, the expectation of constant availability — these are all migraine-hostile by default.

You do not have to fix the entire system, but you can advocate for changes that benefit not just yourself but others in your workplace who may also be affected. Requesting fragrance-free policies, better lighting options, and flexible work arrangements can improve the workplace for everyone, not just people with migraine.

Some larger employers now have neurodiversity or chronic illness employee resource groups. If your company has one, join. If it does not, consider starting one. There is power in numbers.

Self-Employment and Freelance Work

For some people with migraine, self-employment or freelance work offers a better fit than traditional employment. The ability to set your own hours, work from home, and take breaks when needed can significantly reduce migraine-related stress. However, self-employment also means no paid sick leave, no FMLA protection, and the pressure to work even when you are not feeling well because you do not earn unless you produce. If you are considering

self-employment, weigh these trade-offs carefully and plan for financial reserves that can cover your expenses during high-migraine months.

Career Planning with Migraine

Migraine may affect your career choices, but it does not have to limit them. Some people find they need to step back from high-stress, high-stakes roles. Others find that with good accommodation and medication management, they can thrive in demanding careers. There is no right answer — only what is right for you.

If you are early in your career, consider roles and work environments that offer flexibility, good benefits, and accommodations-friendly policies. If you are established in a career and migraine is becoming harder to manage, talk to your doctor about optimizing your treatment before making any drastic career changes. The right medication or accommodation can transform your work experience.

Toolkit Summary

- **Migraine at work involves both absenteeism and presenteeism** — showing up but functioning at reduced capacity carries its own costs.
- **FMLA intermittent leave** provides job-protected, unpaid time off in small increments for migraine attacks. Talk to your doctor about certification.

- **Reasonable accommodations** under the ADA include flexible hours, dim lighting, screen filters, quiet workspaces, fragrance-free policies, and ability to work from home.
- **The disclosure decision is personal.** Consider partial disclosure to your manager and/or HR for protection without making your condition widely known.
- **Build a workplace migraine kit** — medication, glasses, earplugs, cold pack, caffeine, electrolyte powder, backup plan for leaving.
- **After a migraine day, prioritize, communicate briefly, and do not overcompensate.** Recovery matters as much as catching up.
- **Workplace advocacy helps everyone.** Fragrance-free policies, better lighting, and flexible arrangements benefit the whole team.
- **Talk to your doctor** about documentation needed for FMLA or ADA accommodation requests.

ch10

FOR EVERYTHING WE HAVE DISCUSSED ABOUT THE BIOLOGY OF MIGRAINE, THE TRIGGERS, THE MEDICATIONS, AND THE PRACTICAL STRATEGIES, THERE IS A DIMENSION THAT OFTEN GOES UNSPOKEN — THE EMOTIONAL WEIGHT OF LIVING WITH A CHRONIC NEUROLOGICAL CONDITION. MIGRAINE IS NOT JUST A PHYSICAL EXPERIENCE. IT IS A PSYCHOLOGICAL ONE. IT AFFECTS HOW YOU SEE YOURSELF, HOW YOU RELATE TO OTHERS, HOW YOU PLAN YOUR LIFE, AND HOW YOU CARRY THE CUMULATIVE GRIEF OF CANCELED PLANS, LOST OPPORTUNITIES, AND DAYS TAKEN FROM YOU.

In his book *The Body Keeps the Score*, Bessel van der Kolk writes about how the body holds onto trauma long after the event is over. Migraine is not trauma in the clinical sense, but it leaves a mark. The anticipation of the next attack, the time lost to attacks, the relationships strained by cancellations, the career setbacks — all of it accumulates. This chapter is about acknowledging and addressing that accumulation: the grief, the guilt, and the work of rebuilding a relationship with yourself that includes migraine without being defined by it.

Migraine and Mental Health

The connection between migraine and mental health is well-documented and bidirectional. Having migraine increases the risk of depression and anxiety, and having depression or anxiety increases the risk of developing

migraine. They share biological mechanisms — serotonin pathways, inflammation, hypothalamic dysfunction — but the lived experience also plays a role.

Anxiety about attacks. People with migraine often develop what researchers call "anticipatory anxiety" or "migraine fear." The knowledge that an attack could strike at any time — during an important meeting, on vacation, at your child's birthday party — creates a constant low-level vigilance. You scan every sensation in your body for signs of an impending attack. A twinge in your neck, a flash of light in your peripheral vision, a suddenly irritable mood — any of these could be prodrome, or it could be nothing. The uncertainty is exhausting.

This anxiety can become self-fulfilling. The stress of worrying about migraine is itself a migraine trigger, creating a cycle where fear of attacks increases their likelihood.

Depression from lost time. Migraine steals time. If you have three migraine days per month, that is 36 days per year — more than a month of your life spent in pain or recovery. If you have chronic migraine (15+ days per month), you are losing half your life to the condition. This accumulated lost time can lead to a sense of despair, grief for the life you could be living, and depression that is rooted in a real, quantifiable loss.

Substance use is also more common in people with migraine, particularly as people try to self-medicate the pain or the emotional toll. If you find yourself using alcohol or other substances to cope with migraine or its aftermath, talk to a healthcare provider.

The Grief of a Disrupted Life

Grief is an appropriate response to loss, and living with migraine involves real losses. You grieve the plans you cancel. You grieve the career path that became harder or closed because you could not guarantee reliability. You grieve the trips you skipped, the parties you left early, the workouts you missed, the friendships that faded because you said "I cannot, I have a migraine" one too many times.

This grief is not dramatic in the way we typically think of grief. It is quiet and cumulative — a thousand small losses that add up over years.

Cancelled plans. Every time you cancel plans because of a migraine, you disappoint not just yourself but also the people you cancelled on. If this happens frequently, you may start to preemptively decline invitations — "I would love to come, but I cannot promise I will be there" — which protects you from the guilt of cancelling but also isolates you from social connection.

Lost career opportunities. Migraine can affect career trajectories in visible and invisible ways. Visible: missed days, reduced hours, turning down promotions that require more unpredictability. Invisible: the presenteeism that means you are not performing at your best, the networking events you skip, the side projects you cannot take on because you need to preserve energy.

Strained relationships. Partners, family members, and friends may struggle to understand why you cannot "just push through." They may see you cancel plans and interpret it as you not caring enough. They may resent the

unpredictability. The strain is real. Relationships require trust that you will show up, and migraine makes that trust harder to maintain.

Acknowledging the grief is the first step. You cannot heal what you do not name. When you stop pretending that losing 36 days a year to pain is no big deal, you open the door to actually addressing the emotional impact.

Guilt and Self-Blame

"Should have" is the language of guilt in migraine. I should have avoided that trigger. I should have taken my medication earlier. I should have slept more. I should have said no to that glass of wine. I should have been able to prevent this.

This self-blame is understandable — your brain is trying to find control in a situation that feels out of control. If you can figure out what you did wrong, you can fix it next time, and maybe the attacks will stop. But this logic is flawed. Migraine attacks happen even when you do everything right. The threshold model tells us that triggers accumulate, sometimes silently. You cannot control all the variables.

The guilt also extends to other people. "I let everyone down again." "They must be so tired of me canceling." "I am a bad partner, parent, friend." This guilt grows in the absence of honest communication. When you do not explain what is happening, people fill the gap with their own interpretations — and so do you.

The antidote to guilt is self-compassion and communication. You are not failing at migraine. You are managing a difficult neurological condition the best you can. You would not blame a friend for having an asthma attack. Extend the same grace to yourself.

The Identity Shift

Migraine changes how you see yourself. When you have had migraine for years, it can feel like the condition becomes part of your identity. "I am a person with migraine" can shift to "I am a migraine person" — where the condition defines you rather than describing you.

This identity shift has costs. You may stop pursuing activities you love because they "might trigger a migraine." You may stop planning for the future because you cannot trust your body. You may stop seeing yourself as capable, reliable, or ambitious.

The goal of the work in this book — the trigger detective, the medications, the lifestyle changes — is not to eliminate migraine from your life entirely (though that would be wonderful). It is to create enough space between you and the condition that it no longer defines you. The shift I hope for every reader is from "person who has migraine" to "person who manages migraine." That is a fundamental difference. One is a victim of the condition. The other is someone with agency, tools, and strategies.

Therapy Approaches That Help

If the emotional toll of migraine is affecting your quality of life, therapy can be transformative. Not all therapy is equally useful for migraine. The following approaches have evidence for helping with chronic illness and pain:

Cognitive Behavioral Therapy (CBT) for migraine. As mentioned in chapter 7, CBT helps identify and change thought patterns that increase stress and worsen migraine. CBT can also help with the "catastrophizing" that happens during attacks — the spiral from "I have a headache" to "My whole life is ruined and I will never be functional again." By learning to interrupt these patterns, you can reduce the emotional amplification of pain.

Acceptance and Commitment Therapy (ACT). ACT focuses on accepting difficult experiences (including pain and uncertainty) rather than fighting them, while committing to actions that align with your values. This approach is particularly useful for the identity shift described above. ACT helps you live a meaningful life WITH migraine, not just while waiting for migraine to go away.

Pain psychology. Some pain psychologists specialize in working with people who have chronic pain or headache conditions. They combine elements of CBT, ACT, biofeedback, and relaxation training into a tailored approach. If you can find a pain psychologist in your area, they will understand the specific challenges of living with migraine.

Telling Your Story to People Who Don't Understand

One of the most emotionally draining aspects of migraine is constantly explaining yourself to people who do not understand. The "It's just a headache" comments. The "Have you tried drinking more water?" suggestions. The "My cousin had those and she tried acupuncture and it went away" stories.

You do not owe everyone a full explanation of your medical condition. When you choose to explain, keep it simple and direct:

- "I have a neurological disorder that causes severe pain, nausea, and sensitivity to light and sound. It is not a headache."
- "I know you mean well, but that suggestion has not worked for me. I am working with my doctor on this."
- "I cannot make it today because I am having a medical episode. I hope to see you next time."

For your closest relationships — partner, family, close friends — you may want a deeper conversation. Explain the threshold model. Explain that you cannot always predict when an attack will happen. Explain that cancelling is not a choice you make lightly. And ask for what you need: patience, not taking cancellations personally, help during attacks, and space without guilt.

When Relationships Strain

It is worth acknowledging that even with the best communication, some relationships will not withstand the strain of migraine. Partners who cannot handle the unpredictability, friends who take cancellations personally despite repeated explanations, family members who refuse to educate themselves — these relationships may become distant over time. This is painful, but it is also not your fault. Chronic illness reveals the strength of relationships. Some grow stronger. Some do not. Protect your energy for the people who show up for you.

Permission to Feel All of It

This chapter is heavy because the emotional reality of migraine is heavy. You are allowed to be angry about this condition. You are allowed to grieve the time it has taken. You are allowed to be frustrated with a healthcare system that does not always take migraine seriously. You are allowed to be tired of managing it all.

All of these feelings are valid. They do not make you weak or ungrateful. They make you human.

The goal is not to suppress these feelings or pretend migraine is not a big deal. The goal is to feel them, process them, and then — when you are ready — take the next step forward anyway. That is what resilience looks like in chronic illness. Not pretending everything is fine. But living your life fully, with the weight of the condition acknowledged and carried.

Toolkit Summary

- **Migraine and mental health are linked bidirectionally** — migraine increases risk of anxiety and depression, and vice versa. The conditions share biological mechanisms.
- **Anticipatory anxiety** — the fear of the next attack — is common and can itself become a trigger.
- **The grief of a disrupted life is real.** Cancelled plans, lost career opportunities, and strained relationships accumulate. Acknowledge the grief rather than minimizing it.
- **Guilt and self-blame** ("I should have prevented this") are natural but often inaccurate. Extend the same compassion to yourself that you would to a friend.
- **The identity shift** — from "person who has migraine" to "person who manages migraine" — is one of the most important psychological moves you can make.
- **Therapy approaches with evidence:** CBT for migraine (reduces catastrophizing), ACT (acceptance and value-driven action), and pain psychology.
- **Explaining migraine to others:** keep it simple, direct, and set boundaries. You do not owe everyone a full explanation.
- **All your feelings are valid.** Anger, grief, frustration, and exhaustion are appropriate responses to a difficult condition.

- **Talk to your doctor or a mental health professional** if the emotional toll of migraine is affecting your ability to function or enjoy life.

ch11

WE HAVE COVERED A LOT OF GROUND TOGETHER. THE BIOLOGY OF MIGRAINE, THE HORMONAL CONNECTION, THE TRIGGER DETECTIVE WORK, THE MEDICATIONS (ACUTE AND PREVENTIVE), THE FOOD AND NUTRITION, THE SLEEP AND STRESS FOUNDATIONS, THE MOVEMENT STRATEGIES, THE WORKPLACE NAVIGATION, AND THE EMOTIONAL REALITY OF LIVING WITH THE CONDITION. NOW IT IS TIME TO BRING IT ALL TOGETHER INTO A SINGLE, COHERENT PLAN.

This chapter is about the long view. Building your comprehensive migraine management plan, assembling your specialist team, conducting annual treatment reviews, preparing for flares, avoiding the "I thought I was fine" trap, and — most importantly — learning to live fully with migraine as part of your story, not the whole story.

The Comprehensive Migraine Plan

A comprehensive migraine plan has three layers: medication management, lifestyle stabilization, and threshold awareness. Each layer supports the others.

Layer 1: Medication management. This includes your acute treatment strategy (what to take, when to take it, how to handle breakthrough attacks) and your preventive strategy (daily, weekly, or quarterly medication taken to reduce attack frequency). Your medication plan should be written down, reviewed at every specialist visit, and updated when something stops working.

Layer 2: Lifestyle stabilization. This is the foundation work — consistent sleep schedule, stress management practice, regular gentle exercise, adequate hydration, balanced meal timing, and targeted trigger awareness. These are not add-ons to the medication plan; they are essential infrastructure. If the lifestyle foundations are unstable, no medication can fully stabilize your migraine threshold.

Layer 3: Threshold awareness. This is the skill of knowing where your threshold is on any given day and adjusting accordingly. Days when you are well-rested, unstressed, and hormonally stable are high-threshold days — you can take more risks (a glass of wine, a skipped meal, a late night). Days when you are tired, stressed, and in a high-risk hormonal window are low-threshold days — you need to be more protective, avoid stacking triggers, have your acute medication ready, and consider whether you need to adjust your schedule.

A written migraine plan that includes all three layers, reviewed and updated every three to six months, is the most effective tool you can have. It turns your approach from reactive (responding to attacks) to proactive (managing the conditions that create attacks).

Building Your Specialist Team

While your primary care doctor can handle basic migraine care, you will likely need a team of specialists to optimize your management.

Neurologist (or headache specialist). This is your quarterback. They manage your medication strategy, prescribe preventives and acute treatments, and monitor for complications. If you have had migraine for more than a few years and are not seeing a neurologist or headache specialist, consider getting a referral.

Headache specialist. A headache specialist is a neurologist with advanced training in headache disorders. If you have tried several preventives without success, have chronic migraine, or have complicated migraine (hemiplegic, brainstem aura, status migrainosus), a headache specialist is worth seeking out. They will be familiar with the full range of treatment options, including CGRP medications, Botox, nerve blocks, and infusion protocols.

Physical therapist. A PT who understands migraine can help with neck tension, posture, and the musculoskeletal issues that often accompany migraine. Many people with migraine have chronic neck and shoulder tension that feeds into the trigeminal nerve system. Specific manual therapy and exercises for the cervical spine can reduce attack frequency.

Pain psychologist or therapist. As discussed in chapter 10, the emotional toll of migraine is significant. A therapist who works with chronic pain or illness can help you develop coping strategies, address anxiety and depression, and support the identity shift from "person who has migraine" to "person who manages migraine."

Gynecologist or endocrinologist. If hormones are a major factor in your migraine (as they are for many women), a gynecologist who understands the hormone-migraine

connection — or an endocrinologist — can help with birth control decisions, perimenopause management, and HRT considerations. A conversation between your headache specialist and your gynecologist is ideal.

The Annual Treatment Review

Even if your migraine is well-controlled, it is worth doing a comprehensive review at least once a year. Treatments can lose effectiveness over time. Your body changes. Your life circumstances change. And new treatments become available.

An annual treatment review should include:

Medication audit. List every medication you take for migraine (acute and preventive), how often you take it, and how well it works. Are you staying within safe usage limits? Has any medication changed in effectiveness? Are there new medications on the market worth discussing?

Diary review. Review the past three to six months of your migraine diary. Has your attack frequency changed? Are your triggers still the same? Are your attacks more or less severe? Do you have new symptoms?

Life check-in. Have your stress levels, sleep patterns, or hormonal status changed significantly in the past year? Have you started or stopped hormonal contraception? Have you entered perimenopause? Has a major life event (new job, move, loss, relationship change) affected your migraine?

Treatment goals review. What were your treatment goals from last year? Did you meet them? Are they still the right goals? If your current treatment is working well, the goal might be maintenance. If you are still having significant attacks, the goal might be escalation — trying a new preventive, optimizing acute treatment, or addressing a previously overlooked trigger.

New research. Migraine treatment advances rapidly. There may be a new medication, a new device (neuromodulation devices like single-pulse transcranial magnetic stimulation or non-invasive vagus nerve stimulation are available), or a new understanding of triggers that emerged in the past year. Review your approach in light of current evidence.

Bring these findings to your headache specialist appointment. A structured annual review ensures that your treatment does not stagnate.

The Flare Plan

Despite your best efforts, there will be times when your migraine worsens. A flare is a period of increased attack frequency or severity that lasts for weeks or months. Flares can be triggered by life stress, hormonal transitions, weather patterns, medication changes, or sometimes nothing identifiable at all.

A good flare plan helps you respond quickly rather than suffering through.

Step 1: Recognize you are in a flare. Agree with yourself on a definition: if your attack frequency doubles for two consecutive weeks, or if your acute medication stops being effective for a week, you are in a flare. Do not wait three months to acknowledge it.

Step 2: Reduce trigger load. During a flare, your threshold is low. Eliminate any discretionary triggers temporarily. Avoid alcohol, maintain the most consistent sleep schedule you can, prioritize stress management, and do not push your exercise intensity.

Step 3: Optimize acute treatment. Make sure you have enough acute medication on hand. Consider whether you need to use a different medication or combine medications during the flare. Talk to your doctor about whether a short-term bridge treatment (like a course of steroids or an NSAID taper) is appropriate.

Step 4: Revisit prevention. If you are on a preventive, ask your doctor whether the dose can be increased or whether a different preventive should be tried. If you are not on a preventive, a flare is the time to reconsider.

Step 5: Seek support. A migraine flare is not a failure of your management. It is a predictable feature of a chronic condition. Reach out to your healthcare team, your support network, and — if needed — mental health support.

The "I Thought I Was Fine" Trap

One of the most common patterns in migraine management is what I call the "I thought I was fine" trap. You have a period of relative stability — fewer attacks, better response to medication, more confidence. You relax your vigilance. You stay up later, skip a few sleep schedule days, have a few glasses of wine at a party, forget your water bottle, skip a preventive dose. And then the attacks come roaring back, worse than before.

This is not a character flaw. It is human nature. When you feel good, you want to feel normal — and "normal" does not include the constant vigilance that migraine management requires. The trap is that the cost of relaxing vigilance is not immediate. It takes days or weeks for the accumulated effects to trigger a flare. By the time you realize what happened, the threshold is already low, and it takes weeks of disciplined management to bring it back up.

The antidote is not rigid, joyless adherence to every rule forever. The antidote is awareness. Know that when you feel good, you are tempted to drop the foundations. Plan for this. Build accountability — a partner, a migraine support group, a regular check-in with your diary. And when you do slip, recognize it quickly, adjust, and move forward without guilt.

Permission to Adjust Your Life Without Giving It Up

Migraine forces adjustments. There is no way around it. You may need to leave parties early. You may need to decline certain career opportunities. You may need to rest when you would rather be productive. You may need to say no more often than you would like.

These adjustments are not failures. They are adaptations. The goal is not to live the life you would have lived without migraine. That life does not exist. The goal is to build a full, meaningful life within the reality of your condition.

Some of the adjustments you make will be permanent — you will never handle alcohol or sleep deprivation the way people without migraine do. Some will be temporary — during a flare, you pull back; during a stable period, you expand. Some adjustments will evolve as your condition changes.

The poet Rilke wrote, "Let everything happen to you: beauty and terror. Just keep going. No feeling is final." Migraine is not everything that happens to you. It is one thread in the tapestry. The beauty, the relationships, the work that matters, the moments of joy — those threads are there too.

A Final Letter to the Reader

Dear Reader,

I wrote this book because I know what it is like to feel trapped by migraine. I know the frustration of being told "it's just a headache" when your body is in revolt. I know the despair of trying treatment after treatment and feeling like nothing works. I know the grief of canceled plans, lost opportunities, and the quiet erosion of confidence.

I also know that it is possible to build a life that includes migraine without being consumed by it. The tools are imperfect. The journey is not linear. You will have good months and bad months, breakthroughs and setbacks. But the knowledge you have gained from reading this book — the understanding of your brain, the awareness of your triggers, the menu of treatment options, the permission to feel your feelings and ask for what you need — is real power.

You are not broken. You are not weak. You are managing a complex neurological condition in a world not designed for it, and you are doing it with more knowledge and more tools than you had when you started reading.

Migraine is part of your story. It is not the whole story.

Go live your life. Adjust when you need to. Rest when you must. Advocate for yourself fiercely. And never apologize for having a medical condition that you did not choose and cannot control.

You deserve a life that is full, not despite migraine, but with migraine acknowledged, managed, and — as much as possible — set aside so you can focus on what really matters.

With hope, Hannah

Toolkit Summary

- **Your comprehensive migraine plan** has three layers: medication management, lifestyle stabilization, and threshold awareness. Write it down and review it every 3-6 months.
- **Build your specialist team:** neurologist/headache specialist, physical therapist, pain psychologist, and gynecologist/endocrinologist if hormones are a factor.
- **Do an annual treatment review** — medication audit, diary review, life check-in, goals review, new research.
- **Have a flare plan ready:** recognize a flare early, reduce trigger load, optimize acute treatment, revisit prevention, seek support.
- **Watch for the "I thought I was fine" trap** — when things improve, vigilance drops, and attacks return. Plan accountability and adjust quickly without guilt.
- **Give yourself permission to adjust your life.** These are adaptations, not failures. Build a meaningful life within the reality of your condition.
- **Migraine is part of your story, not the whole story.** You are more than your condition. Go live your life.
- **Always talk to your doctor** before starting, stopping, or changing any treatment plan.

resources

THIS CHAPTER PROVIDES QUICK-REFERENCE RESOURCES TO SUPPORT YOUR MIGRAINE MANAGEMENT: A GLOSSARY OF KEY TERMS, COMPARISON CHARTS FOR ACUTE AND PREVENTIVE MEDICATIONS, A TRIGGER TRACKING TEMPLATE, A MIGRAINE-FRIENDLY FOOD LIST, A WORKPLACE ACCOMMODATION LETTER TEMPLATE, A WHEN-TO-GO-TO-THE-ER CHECKLIST, AND A CURATED LIST OF FURTHER READING AND WEBSITES.

Glossary

Allodynia: Pain from a normally non-painful stimulus, such as a light touch, brushing hair, or wearing glasses. Common during migraine attacks due to central sensitization.

Aura: Reversible neurological symptoms that precede or accompany the headache phase in migraine with aura. Can be visual (flashing lights, zigzag lines, blind spots), sensory (tingling, numbness), or speech-related. Usually lasts 5 to 60 minutes.

CGRP (calcitonin gene-related peptide): A neuropeptide released from trigeminal nerve endings during a migraine attack. CGRP causes vasodilation and neurogenic inflammation and is the target of CGRP monoclonal antibodies and gepants.

Chronic migraine: 15 or more headache days per month for at least three months, with at least 8 days meeting migraine criteria.

Cortical spreading depression (CSD): A wave of electrical silence that spreads slowly across the brain cortex, believed to be the physiological basis of migraine aura. CSD activates the trigeminal nerve and initiates the migraine cascade.

Ditan: A class of acute migraine medication targeting the 5-HT_{1F} serotonin receptor, providing pain relief without vasoconstriction. The only available ditan is lasmiditan (Reyvow).

Episodic migraine: Fewer than 15 headache days per month.

Gepant: A class of medications that block the CGRP receptor. Used for acute treatment (rimegepant/Ubrelvy) and prevention (atogepant/Qulipta). Does not constrict blood vessels.

Medication overuse headache (MOH): Headache resulting from frequent use of acute migraine medications. Also called rebound headache. Threshold varies by medication type.

Menstrual migraine: Migraine attacks occurring on days -2 to +3 of the menstrual cycle. Tends to be more severe, longer-lasting, and harder to treat.

Neurogenic inflammation: Inflammation triggered by release of neuropeptides from activated nerve endings, without infection present.

Phonophobia: Abnormal sensitivity to sound during migraine attacks.

Photophobia: Abnormal sensitivity to light during migraine attacks.

Postdrome: The final phase of a migraine attack, after the headache resolves. Often described as a "migraine hangover" with fatigue, cognitive fog, and irritability. Can last up to 48 hours.

Prodrome (premonitory phase): The earliest phase of a migraine attack, preceding headache by hours to days. Symptoms include fatigue, food cravings, yawning, neck stiffness, and mood changes.

Status migrainosus: A migraine attack lasting more than 72 hours despite treatment. Often requires medical intervention.

Trigeminal nerve: The largest cranial nerve, responsible for facial and head sensation. Its activation is central to migraine pain.

Triptan: A class of acute migraine medication (sumatriptan, rizatriptan, eletriptan, zolmitriptan, frovatriptan, naratriptan) that stimulates serotonin receptors, constricts blood vessels, and blocks pain transmission. Contraindicated in uncontrolled hypertension, cardiovascular disease, or history of stroke.

Acute Medication Comparison Chart

Medication Class	Examples	How It Works	Onset	Max Days/ Month	Key Notes
NSAIDs	Ibuprofen, naproxen, diclofenac, ketorolac	Reduces neurogenic inflammation	30-60 min	14	First-line for mild-moderate pain; avoid in patients with GI/kidney disease or on blood thinners
Triptans	Sumatriptan, rizatriptan, eletriptan, zolmitriptan, frovatriptan, naratriptan	Serotonin receptor agonist; vasoconstriction; blocks CGRP	15-60 min	8-10	Contraindicated with uncontrolled HTN, CAD, stroke; injectable sumatriptan fastest
Gepants	Rimegepant (Ubrovelvy)	Blocks CGRP receptor	30-60 min	18	No vasoconstriction; safe for patients with cardiovascular disease; expensive
Ditans	Lasmiditan (Reyvow)	5-HT1F receptor agonist	30-60 min	8-10	No vasoconstriction; causes drowsiness; avoid driving for 8 hours
Antiemetics	Ondansetron, prochlorperazine, metoclopramide	Blocks nausea signals	15-45 min	As needed	

Medication Class	Examples	How It Works	Onset	Max Days/ Month	Key Notes
					Use along with other acute meds when nausea is present

Preventive Medication Comparison Chart

Medication	Type	Dosing	Common Side Effects	Key Points
Erenumab (Aimovig)	CGRP mAb	Monthly injection	Injection site reaction, constipation	Blocks CGRP receptor
Galcanezumab (Emgality)	CGRP mAb	Monthly injection	Injection site reaction	Binds to CGRP
Fremanezumab (Ajovy)	CGRP mAb	Monthly or quarterly injection	Injection site reaction	Binds to CGRP
Epitnezumab (Vyepti)	CGRP mAb	Quarterly IV infusion	Infusion reactions	Given in medical office
Atogepant (Qulipta)	Oral gepant	Daily pill	Nausea, constipation	

Medication	Type	Dosing	Common Side Effects	Key Points
				Oral alternative to injectable CGRPs
Botox	Neurotoxin	31 injections every 12 weeks	Injection pain, neck stiffness	For CHRONIC migraine only (15+ days/month)
Propranolol	Beta blocker	Daily pill	Fatigue, dizziness, cold hands	May lower blood pressure; avoid in asthma
Amitriptyline	Tricyclic antidepressant	Daily pill (low dose)	Drowsiness, dry mouth, weight gain	Take at bedtime; doses lower than for depression
Topiramate (Topamax)	Antiseizure	Daily pill (slow titration)	Cognitive slowing, tingling, kidney stones	Pregnancy warning
Candesartan		Daily pill	Dizziness	

Medication	Type	Dosing	Common Side Effects	Key Points
	ARB (blood pressure)			Fewer side effects than many alternatives

Trigger Tracking Template

Use this template daily (not just on attack days).

Date: _____

Migraine today? Yes / No If yes: Onset time: ____ Severity (1-10): ____ Pain location: Left / Right / Both / Entire head Symptoms: Nausea / Vomiting / Light sensitivity / Sound sensitivity / Aura / Other: ____

Sleep: Hours slept: ____ Quality (1-5): ____ Same wake time as usual? Yes / No

Meals: Breakfast: ____ Lunch: ____ Dinner: ____ Snacks: ____ Skipped any meal? Yes / No

Hydration: Glasses of water: ____

Caffeine: Type: ____ Amount: ____ Time: ____

Alcohol: Type: ____ Amount: ____ Time: ____

Triggers present: ☐ Stress (level 1-5: ____) ☐ Menstrual cycle (day: ____) ☐ Weather change ☐ Neck tension ☐ Screen time (hours: ____) ☐ Strong smells ☐ Intense exercise ☐ Fatigue ☐ Bright/flickering lights ☐ Missed/delayed meal ☐ Specific food: ____

Physical activity: Type: ____ Duration: ____ Intensity:
Low / Moderate / High

Medication taken: Acute: ____ Dose: ____ Time: ____
Relief: None / Partial / Significant / Complete Preventive: ____
Dose: ____ Time: ____

Notes: _____

Migraine-Friendly Food List

Vegetables: Leafy greens (spinach, kale, arugula), broccoli, cauliflower, bell peppers, zucchini, cucumber, carrots, sweet potatoes, asparagus

Fruits: Berries (blueberries, strawberries, raspberries), bananas, apples, pears, melon, cherries

Proteins: Fresh fish and seafood (salmon, sardines, mackerel), chicken, turkey, eggs, tofu, lentils, chickpeas, beans

Grains: Oats, brown rice, quinoa, whole wheat, buckwheat

Dairy: Fresh cheeses (mozzarella, cottage cheese, cream cheese), yogurt, milk (if tolerated)

Fats: Olive oil, avocado oil, avocado, nuts and seeds (almonds, walnuts, flaxseeds, chia seeds)

Beverages: Water, herbal tea (chamomile, peppermint, ginger), diluted fruit juice

Herbs and spices: Ginger, turmeric, basil, oregano, rosemary, cilantro (avoid large amounts of garlic and onion if they are personal triggers)

Workplace Accommodation Letter Template

[Date]

To Whom It May Concern:

[Patient name] is under my care for migraine, a neurological disorder recognized as a disability under the Americans with Disabilities Act (ADA) and similar applicable laws.

Migraine is characterized by recurrent episodes of moderate to severe headache accompanied by nausea, vomiting, and sensitivity to light, sound, and smell. Attacks can last 4 to 72 hours and can be severely disabling during the acute phase.

Based on my patient's medical condition, I recommend the following accommodations:

1. Flexible start and end times to accommodate unpredictable attacks
2. The ability to work from home during attacks when the condition permits
3. Permission to use a personal desk lamp in lieu of overhead fluorescent lighting
4. Installation of blue-light filters or anti-glare screens on computer monitors
5. Relocation to a quieter workspace away from high-traffic areas
6. Permission to take short breaks in a quiet, dark space when symptoms escalate
7. Fragrance-free workspace policy due to odor sensitivity

These accommodations are based on current medical evidence for migraine management and are consistent with standard medical recommendations for this condition.

Please contact my office if you require additional information.

Sincerely,

[Doctor's name, credentials] [Practice name and contact information]

When-to-Go-to-ER Checklist

Seek emergency medical attention if you experience any of the following:

☐ The headache came on suddenly and reached peak intensity in seconds or minutes ("thunderclap headache") ☐ The headache is the worst headache of your life ☐ You have new neurological symptoms you have never experienced before (weakness on one side, difficulty speaking, vision loss, confusion, seizures) ☐ Your migraine attack has lasted more than 72 hours without relief (status migrainosus) ☐ You are vomiting repeatedly and cannot keep down fluids or medications ☐ Your migraine is accompanied by fever, stiff neck, or rash ☐ You have had a head injury in the past 24 hours ☐ You are on blood-thinning medication and have a severe headache

If in doubt, seek medical evaluation. It is better to be checked than to stay home with a potentially serious condition.

Further Reading and Websites

American Migraine Foundation <https://americanmigrainefoundation.org> Comprehensive patient education, a "Find a Clinician" tool, and research updates. Run by leading headache specialists.

Miles for Migraine <https://milesformigraine.org> Advocacy, awareness, and community-building for people with migraine and their families. Educational events and fundraising walks.

Migraine.com <https://migraine.com> Online community and information hub with patient stories, treatment updates, and practical tips. Content is medically reviewed.

The Migraine World Summit <https://migraineworldsummit.com> Annual online event with interviews from leading headache specialists. Past interviews available on demand.

CGRP-Focused Neurologist Directories - American Headache Society: <https://americanheadachesociety.org/resources/find-a-headache-specialist> - American Migraine Foundation: <https://americanmigrainefoundation.org/resource-library/find-a-clinician>

Further Reading

The resources listed above are excellent starting points for ongoing education, community, and advocacy. Bookmark your favorites, revisit them as your understanding of your own migraine evolves, and share them with family members or

colleagues who want to understand your condition better. Knowledge is power in migraine management, and the landscape is always advancing.

About the Author

AUTHOR BIO GOES HERE

Also by AUTHOR_NAME

This is her first book.

